



Situation Analysis

LEBANON CRISIS

Period: 08/06/26 – 14/06/26

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Introduction

Purpose

This report provides a comprehensive overview of the current humanitarian situation, focusing on both operational environment factors and sectoral analysis. It aims to support informed decision-making for humanitarian partners and stakeholders.

About GANNET

GANNET is a Generative AI Initiative from Data Friendly Space, offering AI-powered tools built to make knowledge and real-time information accessible to our community in a resource-efficient and ethical way. It provides tools such as the GANNET Virtual Assistant and GANNET SituationHub to help humanitarian actors access, analyze, and utilize critical information for their operations.

GANNET SituationHub

The GANNET SituationHub is a generative AI-powered digital public good designed to identify and analyze humanitarian data from thousands of trusted sources in real time. By transforming complex data into clear insights, it enables faster, more effective response strategies and improved coordination among stakeholders.

GANNET Virtual Assistant

The GANNET Virtual Assistant is a verified chatbot designed for humanitarian quick decision-making. Built on cutting-edge Generative AI and trained with humanitarian expert-reviewed data, it provides real-time information. The GANNET Virtual Assistant facilitates rapid data access and synthesis for crisis response.

Methodology Overview

The Situation Hub's analytical process uses a structured Human-in-the-Loop workflow that combines automated analysis retrieval through GANNET Virtual Assistant, DFS's AI-powered humanitarian assistant, with expert review by the DFS analysis team. GANNET automates the extraction and generation of crisis-related insights based on structured prompts, which are aligned with a tailored analytical framework. These outputs are then reviewed, validated, and refined by analysts to ensure accuracy, contextual relevance, and operational utility. The workflow is organized into four key phases: planning and setup, ongoing operations, quality assurance, and system evolution, ensuring a balance between speed and analytical rigor.

Planning, Setup & Quality Control

In the planning phase, analysts develop a contextualized analytical framework, largely based on the Joint Intersectoral Analysis Framework (JIAF), and convert it into structured prompts for AI-assisted data extraction. These prompts are refined over time to improve output quality. Data sources are vetted for reliability, and translations are reviewed for accuracy. Using a centralized Content Management System (CMS), analysts review and curate AI-generated outputs, validate them against external sources, and make refinements to enhance clarity and relevance. The CMS enables collaborative editing and version tracking, ensuring outputs align with humanitarian needs.

Bias Mitigation

To minimize bias, the team diversifies data sources, cross-verifies information, and continuously evaluates the AI's performance. Analysts are crucial in identifying and correcting inaccuracies, especially in automated categorization. Future improvements aim to enhance source credibility checks and sentiment analysis, further strengthening the reliability of the Situation Hub's outputs.

Crisis Overview

The SituationHub landing pages use a standardized yet customizable template to deliver consistent, comprehensive crisis overviews. These pages integrate static content with dynamic data from sources like ACLED and DTM via open APIs, enabling interactive visuals with real-time updates. Regular reviews and verification with in-country partners ensure accuracy, while manual updates are used for sources not connected to APIs.

Media Monitoring

The media monitoring process has evolved from a manual, list-based approach to an AI-assisted system integrated into the CMS. This allows analysts to efficiently review, filter, and contextualize media narratives, while automation helps track emerging trends.

Overview

The following overview has been generated using the information available up to June 26, 2026. It provides a synthesized summary and key insights into the crisis based on the most recent data accessible at that time.

Summary

The conflict between Hezbollah and Israel, which began in October 2023 and spiked in September 2024, entered a new phase in early 2026. Although a November 2024 ceasefire was put in place, persistent daily violations and Israel's continued occupation of five positions in southern Lebanon blocked reconstruction and the return of displaced residents. The truce officially collapsed on March 2, 2026, following U.S. and Israeli strikes on Iran, which triggered retaliatory Hezbollah rocket and drone attacks, subsequent heavy Israeli bombardments, expanded ground operations, and widespread evacuation orders across southern Lebanon and Beirut's southern suburbs.

The conflict between Hezbollah and Israel rooted in late 2023 and early 2024 entered a volatile new phase on March 2, 2026, when a fragile November 2024 truce collapsed following U.S. and Israeli strikes on Iran. Hezbollah's retaliatory rocket and drone attacks prompted a massive Israeli response, shifting from "limited" incursions into an intensified ground campaign and heavy bombardments that triggered widespread evacuation orders across southern Lebanon and Beirut's southern suburbs. Amid this escalation, UNIFIL has faced severe operational constraints in its final mandate year; with peacekeepers killed, patrols targeted, and infrastructure damaged, the mission's ability to monitor and document violations along the Blue Line has been deeply compromised.

UNIFIL has faced severe operational constraints during its final mandate year. Since March, peacekeepers have been killed, patrols targeted, and monitoring infrastructure damaged, undermining the mission's ability to document violations along the Blue Line. Israeli military operations intensified throughout March and May 2026, evolving from "limited" operations into a broader ground campaign extending deeper into southern Lebanon.

Following the massive military escalation that began in early 2026, Israel has targeted thousands of sites across Lebanon. While a breakthrough in early June led to a US - brokered ceasefire agreement outlining "pilot zones" for Lebanese army control, the diplomatic framework remains highly fragile. Hezbollah officially rejected the truce, demanding a full Israeli military withdrawal, while ongoing localized violence including an IDF strike on Beirut's southern suburbs and a major Hezbollah rocket barrage on June 7 continues to threaten a total collapse of the ceasefire, leaving over 1 million civilians displaced amid climbing casualties.

Between April and June 2026, Lebanon's humanitarian crisis deepened severely. The Ministry of Public Health reported that casualties since March climbed to over 3,613 killed and 11,072 injured, with women, children, and the elderly making up more than a third of the civilian toll. Continuous airstrikes flattened neighborhoods, schools, and nearly 200 healthcare facilities across Southern Lebanon, the Bekaa Valley, and Beirut's southern suburbs, keeping 1.2 million people displaced and completely overwhelming shelters in cities like Tyre and Saida.

Humanitarian funding in Lebanon remains critically inadequate. On June 5, 2026, the UN and the Lebanese government drastically expanded their initial response strategy, launching a revised Flash Appeal that raised the total financial requirement to \$639.9 million to support 1.4 million people through August. However, with only \$190 million of this total secured, massive funding gaps persist. Aid agencies warn that imminent pipeline and service breaks starting in July will directly threaten vital food, health, clean water, and shelter assistance for over 1.2 million displaced people.

Key Insights

1. The resumption of large-scale hostilities on 2 March 2026 marked the most severe deterioration in Lebanon's security since the November 2024 ceasefire. The escalation followed months of repeated ceasefire violations, with UNIFIL documenting over 10,000 air and ground breaches and at least 331 deaths before fighting fully resumed. Since March 2026, over 3,070 people have been killed and 9,362 injured, with women, children, and the elderly accounting for an estimated 25% to 35% of the total civilian casualties, while more than 1.2 million people have been displaced. While Israel has not launched a single, massive 24-hour campaign as concentrated as the April 8 wave, it has maintained a relentless, daily bombardment strategy throughout April and May 2026.

2. A diplomatic process began after the 8 April 2026 strikes, marking the first high-level Israel-Lebanon engagement since 1993. Preparatory talks on 14 April 2026 at the U.S. State Department, led by Secretary of State Marco Rubio, revealed opposing positions. Israel demanded Hezbollah's disarmament, stronger enforcement of UN Resolution 1701, and security guarantees for northern Israel, while Lebanon called for an end to Israeli airstrikes, withdrawal from southern Lebanon, release of detainees, return of displaced civilians, and reconstruction aid. A second round on 23 April 2026, chaired by President Donald Trump, led to a breakthrough in talks on 14-15 May 2026, with agreement on a 45-day ceasefire extension and a roadmap for further negotiations. However, Hezbollah rejected the process as a "concession" to Israel and continued cross-border attacks during negotiations.

3. The conflict is heavily rooted in the wider 2026 U.S. Iran war, but Lebanon's diplomatic fate has recently decoupled from it. While the disputed April 7 U.S. Iran truce initially left Lebanon highly vulnerable to regional escalation, the resulting crisis pushed Washington to mediate direct, high-level talks between Israel and the Lebanese government. This parallel track achieved a major breakthrough on May 15, 2026, securing a 45-day ceasefire extension and a structured military political roadmap. While the broader U.S. Iran negotiations remain deadlocked, Lebanon has carved out its own distinct diplomatic track, though the ground reality remains highly volatile due to isolated cross-border skirmishes and Hezbollah's rejection of the bilateral process.

4. Between 2 March and mid-May 2026, the Lebanon conflict displaced over 1.2 million people, forcing 20% of the population (1 million internal refugees) from their homes and sending 300,000 fleeing into Syria. Although a mid-April truce was extended for 45 days in mid-May, returnees were immediately forced back into displacement after finding their border villages structurally leveled and cut off

from water and electricity. Humanitarian agencies are stretched to their limits; the UN's \$308.3 million Flash Appeal is only 51.3% funded by late May, severely crippling the response framework as needs continue to surge.

5. Food insecurity was already projected to worsen before the renewed escalation. A joint FAO/WFP/Government IPC analysis found that approximately 874,000 people (17% of the population analyzed) faced acute food insecurity between November 2025 and March 2026, with projections indicating a further rise to approximately 961,000 people, or 18% of the population, between April and July 2026. The renewed conflict, which has cut humanitarian access and re-displaced more than 1.2 million people, is expected to sharply exceed those projections.

6. The March 2026 escalation has significantly reversed displacement gains and risks conflating two fundamentally different population movements. Before 2 March, Lebanon was experiencing the largest Syrian voluntary return movement since the conflict began: UNHCR inactivated 552,413 Syrian cases due to confirmed or presumed return since January 2025, underpinned by Lebanon waiving exit fees and re-entry bans from July 2025. Since the resumption of hostilities, more than 450,000 people in total, including nearly 200,000 Syrians and over 28,000 Lebanese, have fled across the border under bombardment. UNHCR's ability to ensure returns are safe, informed, and truly voluntary is therefore severely compromised due to the active conflict environment. With Lebanon hosting approximately 1.3 million Syrian refugees, the highest per capita globally, and conditions in Syria remaining fragile, the March escalation threatens to lock a deeply vulnerable population into a cycle of compounded, potentially irreversible displacement with no viable protection framework on either side of the border.

7. Lebanon's economy had registered its first growth since 2017 — the World Bank recorded real GDP expansion of 3.5% in 2025 — but the March 2026 escalation has placed that fragile recovery in severe jeopardy. The IMF's April 2026 World Economic Outlook notes that conflicts generate large and persistent output losses exceeding those from financial crises or natural disasters, and Lebanon enters this escalation with a banking sector carrying over USD 72 billion in accumulated losses and public debt at 176.5% of GDP. The conflict has directly struck the pillars of the 2025 recovery: the south of the Litani has been fully severed from the rest of the country since 16 April, collapsing market activity and cutting supply chains entirely; the spring tourism season is effectively lost; and the escalation delays IMF reform requirements, deters investment, and risks prolonging stagnation well into 2026 and beyond. Lebanon's path to an IMF program, of which only the bank secrecy amendment has been adopted from 8 required prior actions, is now contingent on a peace that does not yet exist.

Economy

Current Economic Situation

Lebanon remains in sovereign default with a persistent banking crisis, blocked or amputated deposits, and a Moody's credit rating of 'C' in the absence of debt restructuring and a comprehensive stabilization programme [6][1]. War-related economic damage is substantial, with direct and indirect losses since 2024 estimated at \$20–25 billion, and public revenues have collapsed – war-related losses cut government income by roughly half, with the 2026 estimated income tax of \$644 million accounting for about 11% of public revenue [1][14]. The balance of payments is under acute pressure from high import dependence (energy, medicines, food) and plunging tourism: total passengers fell 34.2% and arrivals fell 40.2% in the first five months of 2026, reducing essential foreign-exchange inflows [6][1]. Sharp price shocks are eroding purchasing power, with fuel prices rising approximately 84% between mid-February and mid-May 2026, food prices 10–20% higher by mid-May 2026 versus 2025, and bread prices increasing approximately 15% following the March escalation [5]. The industrial sector is contracting sharply – with reported declines of around 45–50% – alongside factory closures in insecure areas and heavy export disruptions, while agriculture, construction and industry jointly represented about 16% of GDP in 2024 and services (education and health) about 19% of GDP, underscoring broad sectoral damage to output and human capital [20][1][6]. Authorities and international actors have discussed rapid IMF financing of up to \$1 billion for budgetary and humanitarian support, and calls have been made for tax relief and fiscal measures to support the private sector, but such emergency funding would not replace the need for debt restructuring and full financial sector remediation [6][1][14]. The unresolved banking crisis and lack of a credible recovery path continue to limit credit, investment, and Lebanon's ability to absorb and finance reconstruction needs [6][1].

Post-Conflict Economic Landscape

Lebanon faces a massive reconstruction bill estimated at approximately USD 20–25 billion in war-related direct and indirect losses since 2024, with more than 61,000 housing units totally or partially damaged during March–early May 2026 [1][6]. A UN-led assessment by UNDP found around USD 365 million in building damage in Beirut and Mount Lebanon, estimating approximately 649,000 cubic metres of debris, 146 buildings completely destroyed, 264 partially destroyed, 3,168 housing units destroyed and 4,437 damaged in that area [10]. UNOPS estimated Lebanon generated 14.5 million metric tonnes of conflict debris in 2024 and promotes circular rubble-reuse approaches to reduce material costs, create local jobs and accelerate recovery [21][37]. Critical infrastructure damage is constraining recovery: key bridges and roads – including Litani crossings – have been destroyed or damaged, isolating over 150,000 people and restricting humanitarian access, while the health system has been eroded by repeated attacks (WHO recorded 196 attacks since 2 March, with 131 deaths and 379 injuries) and education infrastructure has been widely disrupted [5][10]. Southern Lebanon, estimated to account for approximately 25% of national economic activity, has concentrated a major part of the economic shock, with agriculture, construction and industry – which together represented about 16% of GDP in 2024 – hit particularly hard; assessments report more than 22% of agricultural land damaged and 76% of farmers in southern Lebanon affected by evacuation orders and disruptions to farming activities [1][6][5]. The private and industrial sectors report steep declines – some industry observers cite production contractions of roughly 45–50% in affected segments – with factory closures in insecure areas, sharp export reductions and substantial job losses among daily and seasonal workers [20]. Humanitarian financing and immediate recovery funding are severely short: the revised 2026 Flash Appeal (March–August) increased needs to USD 639.9 million for an estimated 1.4 million people, while only USD 185.9 million had been received by 31 May, leaving a funding gap of approximately USD 454 million [5]. Oxfam's digital needs assessment found livelihoods overwhelmingly the top priority (47.12% of respondents), with displaced households reporting livelihoods as their primary need more frequently than other groups [25]. Return and reintegration prospects remain weak: monitoring in neighbouring Syria and returnee assessments show limited immediate employment opportunities – 51% of assessed locations reported no immediate work – with continued reliance on savings, remittances and unsustainable coping mechanisms among returned or displaced households [15][80]. Recovery will require large-scale international finance and clear debt resolution, coordinated reconstruction planning, and transparent, people-centred approaches, as emergency IMF financing and current humanitarian aid levels are insufficient by themselves to meet the scale of physical reconstruction and socio-economic recovery documented to date [1][6][5].

Stability of Currency and Bank System

The Banque du Liban is described as able to preserve apparent monetary stability but lacks the capacity to finance large-scale reconstruction without creating new macroeconomic imbalances; using foreign currency reserves for reconstruction imports would risk further eroding those reserves and repeating past confusion between monetary policy, public financing and bank rescue [1]. Apparent exchange-rate stability has coexisted with reserve losses and a lack of implemented reforms, underscoring that restoring central-bank credibility, completing debt restructuring, unifying accounts and strengthening governance are prerequisites for sustainable monetary stability and banking-sector recovery [6][1]. Credit rating agency Moody's assigns Lebanon a 'C' rating, highlighting prolonged sovereign default, an under-restructured banking sector, lack of credible debt-restructuring and governance reforms, and noting that limited emergency IMF financing – possible rapid support of up to about USD 1 billion – would not substitute for a comprehensive stabilization programme [6]. The fiscal and liquidity response space is constrained: prolonged economic crisis has depleted state resources and weakened national response capacity, estimated income tax receipts for 2026 are reported at USD 644 million (about 11% of public revenue), and authorities have signalled using part of existing fiscal margins to support the economy in the short term [5][14]. Domestic price shocks are increasing inflationary and fiscal pressures, with food prices rising about 10–20% by mid-May 2026 versus 2025, fuel prices climbing approximately 84% between mid-February and mid-May 2026, and bread prices rising approximately 15% after the March escalation [5].

Employment

Livelihoods represent the most critical priority for affected populations, with Oxfam's digital needs assessment finding households reporting loss of income and an affordability crisis covering rent, food, medication and other essentials [25]. Availability of unskilled day work has declined sharply: casual labour fell by 22% to an average of 1.8 days per week, while the purchasing power of a day's wage for basic food is about 30% below 2025 levels and approximately 20% below the three-year average [69]. Continued large-scale displacement — nearly one million people displaced by early June — has further disrupted livelihoods and market access, with migrants and non-Lebanese groups reported as disproportionately affected by loss of formal accommodation and income opportunities [34][23]. The least mobile productive sectors — agriculture, construction and manufacturing, together about 16% of GDP in 2024 — have been particularly disrupted; around 22% of agricultural land had been damaged and evacuation orders affected an estimated 76% of farmers in southern Lebanon, undermining rural employment and seasonal labour opportunities [6][5]. The crisis has a gendered employment impact: women face reduced access to income-generating opportunities due to childcare, mobility and safety constraints, and women-headed households report particular difficulty covering basic costs [25][12]. Humanitarian actors are prioritising cash responses to protect livelihoods, with planned multi-purpose cash assistance and government-shared beneficiary lists for emergency MPCA/E-MPCA distributions covering tens of thousands of households to mitigate income loss where markets remain functional [34][16].

Politics

Governance Structures and Rule of Law

Lebanese leaders have publicly framed a choice between restoring a state monopoly on arms and remaining subject to militia influence, with President Joseph Aoun and Prime Minister Nawaf Salam calling for the exclusivity of arms and for the Lebanese army to assume central roles in the south [164][166][143]. Foreign Minister Youssef Rajji has similarly argued the state can resolve Hezbollah's weapons issue if appropriate decisions and international support are provided, while noting the government lacks the unilateral political and military means to impose disarmament [165][143]. Analysts describe this as a governance dilemma in which Lebanon's legal sovereignty coexists with limited state control over armed actors, producing a national-legal versus regional-military tension in decision-making and accountability [143][165].

Reconstruction and recovery have been explicitly conditioned on stronger governance and transparency, with calls including publishing a damage map, clear compensation criteria, traceable aid, municipal involvement under oversight, and phased programming to reduce the risk of clientelist reconstruction [131]. Credit-rating and economic analyses warn that sovereign default, unresolved banking sector losses, and the absence of credible budgets, audits, and legal reforms undermine the state's capacity to finance and execute reconstruction and to restore public-sector functionality [119]. Sectoral reforms are being presented as institutional tests — ending Middle East Airlines' monopoly, for instance, is framed as a test of regulator independence requiring a professional, financed civil aviation authority able to enforce safety, licensing, and equal access rather than ad hoc political licensing [152]. Humanitarian coordination planning includes an IOM Displacement Tracking Matrix (DTM) to monitor displacement and inform decision-makers for March–August 2026, for which IOM requested USD 1,240,000 to sustain DTM operations [156].

Political Freedoms and Recent Laws

Intense fighting since 2 March 2026 has killed thousands and displaced over one million people in Lebanon; by 17 April, reports recorded 2,294 deaths and 7,544 injured, putting severe strain on civic space and public services [153][149][173]. Large-scale displacement has heightened communal and sectarian polarisation and fuelled resentment toward displaced communities, increasing the risk of harassment, discrimination, and localised attacks that undermine freedom of assembly, association, and safe civic engagement [110]. Humanitarian and governance gaps — including the government's inability to fully manage internal displacement, political sensitivity around IDP information, and administrative bottlenecks delaying response — have removed a state-led information base and complicated independent monitoring and civic oversight [110].

The UN has called for an immediate diplomatic settlement and protection of civilians, with Secretary-General António Guterres highlighting the scale of displacement and urging ceasefire commitments, while the UN human rights office announced a mission to investigate alleged violations in Lebanon [173][184]. Lebanon's political process has been the focus of intensive external diplomacy, with US-sponsored Lebanese-Israeli negotiations set to resume in Washington and the US ambassador meeting President Joseph Aoun and Prime Minister Nawaf Salam to discuss the process [178]. Public debate on constitutional change — including renewed calls for federalism to decentralise governance and deepen local political participation — has appeared in Lebanese commentary and media [142]. European governments are also factoring developments on the Lebanese front into diplomatic and sanction calculations regarding Israel, a dynamic that could influence regional accountability and political pressures on actors affecting Lebanon's civic space [174].

Influence of Non-State Actors and Political Factions

Hezbollah's operational link to Tehran has become a primary channel for negotiations on the Lebanese file, with Parliament Speaker Nabih Berri acting as a privileged relay conveying Hezbollah positions to external mediators, and the Iranian ambassador reportedly remaining in Beirut despite a government recall — signs that non-state actors and external patrons can bypass official diplomacy and narrow Beirut's diplomatic autonomy [143][190]. The Lebanese government's formal steps, such as ordering the Iranian ambassador to leave, have had limited practical effect when issues directly affect Hezbollah, reducing the state's ability to assert an independent reading of the crisis [143][190]. Fragile ceasefires and continued strikes — including attacks on Dahiyeh and incidents against security forces and convoys — have weakened confidence in mediation and reinforced arguments within Hezbollah and Tehran that the Lebanese front must remain part of broader regional negotiations, complicating efforts to dissociate state decision-making from militia-linked actors [143][186].

The politicisation of displacement and aid has increased local tensions: displaced groups are often perceived as affiliated with armed actors, fuelling sectarian and political polarisation and raising risks of discrimination, harassment, and localised attacks against displaced people [110]. Humanitarian operations are constrained by insecurity and notification-based access mechanisms in the south and other hard-to-reach areas, even as partners and mediators emphasise placing the Lebanese army at the centre of stabilisation despite heavy operational and political pressures on the military [105][144][143][192]. These dynamics have coincided with a sharply increased humanitarian burden and funding shortfall – the revised Flash Appeal (4 June 2026) raised people in need to 1.4 million and total requirements to USD 639.9 million, with only USD 185.9 million received by 31 May, leaving an estimated funding gap of approximately USD 454 million – further eroding state and humanitarian capacity amid contestation over non-state actor roles [110].

Public Trust in State Institutions

Prime Minister Nawaf Salam reiterated on 13 June 2026 that only the State negotiates on Lebanon's behalf and called for consolidation of state authority and the monopoly on the use of force, including deployment of the Lebanese armed forces in conflict-affected areas [192]. Reports and analysts highlight that state authority remains partial – the government does not control all weapons on Lebanese territory, and key intermediaries such as Nabih Berri and Hezbollah continue to mediate with Iran, undermining Beirut's diplomatic autonomy and its ability to enforce disarmament or prevent strikes [143]. Public and political scepticism is evident: Hezbollah figures have criticised government concessions and argued for continued resistance, and incidents such as the Ghobeiry raid have been interpreted as demonstrating that military decisions often occur outside Beirut's control, reinforcing doubts about the state's effectiveness in protecting citizens and asserting sovereignty [198][154].

Role of Marginalized Groups

An Oxfam Lebanon Digital Needs Assessment conducted in April–May 2026 found that the crisis is gendered: women and girls face heightened protection risks including lack of privacy, harassment, exploitation, intimate partner violence, unsafe WASH access, and psychosocial distress, while men and boys face pressures from income loss, debt, provider expectations, and mobility restrictions [149][153]. The assessment also flagged differential impacts for people from diverse Sexual Orientation, Gender Identity and Expression and Sex Characteristics (SOGIESC) groups and for persons with disabilities, and will inform Oxfam's response design and targeting [149]. UNFPA's Lebanon Flash Appeal (March–August 2026) called for targeted demographic vulnerability assessments to identify and map the needs of older persons and persons with disabilities, prioritised youth engagement through resourcing youth-led initiatives, training, psychosocial support and GBV prevention, and strengthened GBV coordination co-chaired by UNFPA, the Ministry of Social Affairs, and UNHCR to coordinate services and referrals in collective shelters [139]. Reporting in June 2026 noted that the political impasse – including Hezbollah's rejection of direct talks and the absence of a government roadmap – risks prolonged or permanent displacement among Shi'a communities if the group remains on a war footing [203].

Security And Stability

Security Overview

Renewed hostilities between Israel and Iran on 7–8 June increased regional instability, and Lebanon has experienced continued instability, renewed strikes, evacuation notices, and access constraints that are disrupting health services and response operations [253][256]. More than one million people in Lebanon have been displaced since the escalation in March 2026, with Lebanese government-linked figures citing over 3,600 killed and thousands injured during the period [223][271][255][265][264]. WHO has warned of acute health threats in affected areas – including high trauma caseloads, communicable disease risks linked to displacement, rising measles, and disruptions to essential services due to fuel, power, and supply-chain constraints [253]. UN Secretary-General António Guterres has called for urgent de-escalation, noting the fragile ceasefire environment and the risk of broader regional spill-over that is complicating humanitarian access and protection of civilians [271][263].

Security Forces and Law Enforcement

Lebanese security forces have been directly affected by the conflict since 2 March: ACLED recorded at least 21 Israeli strikes on Lebanese security forces that killed about 30 personnel and injured 17, with Israeli drones targeting identified LAF vehicles, undermining state institutions' operational capacity and morale [225]. In early June, an Israeli strike hit a Lebanese Army vehicle operating beyond the so-called "yellow line", killing three soldiers and prompting Lebanese military sources to dispute Israeli claims that the area required coordination with the ceasefire mechanism [225]. The expected expansion of Lebanese Armed Forces deployments in southern areas remains politically sensitive and operationally risky: mediators have pushed to place the Army at the centre of stabilization efforts, but observers note this exposes the institution to heavy pressure and casualties while it lacks the capacity to enforce disarmament of non-state armed groups on its own [218]. Attacks have also affected first responders and emergency services, including critical damage to Jabal Amel Hospital on 2 June and emergency responders injured while assisting victims, further complicating protection of civilians and operational emergency response [253][247]. Humanitarian access and civil-military coordination remain constrained and heavily regulated by notification mechanisms, with 203 humanitarian movements supported through the Humanitarian Notification System between 2 March and 11 June – 170 of them south of the Litani – and multiple missions cancelled or postponed due to security concerns between 26 May and 9 June [242]. On 10 June, the UN human rights office announced a deployment of investigators to Lebanon to assess potential violations of international law by all parties during the conflict, signalling international scrutiny of conduct by state and non-state security actors [233].

Demography

Demographic Overview

Since 2 March 2026, more than one million people have been internally displaced in Lebanon, equal to about 20% of the country's population, according to UNFPA (data as of 1 June 2026) [294]. As of 11 June 2026, 134,800 displaced people (35,700 families) were registered in 642 collective shelters, with shelters in Beirut and Mount Lebanon at or near capacity and North governorate emerging as a primary destination for newly displaced families [290][304]. Most displaced people remain outside formal sites in host communities, informal settings or open spaces, with major arrival and shelter pressure reported in Saida, Tyre, Beirut's southern suburbs, Bekaa and northern areas such as Akkar and Tripoli [304][306][295].

A digital needs assessment (DNA) conducted mid-April to end-April 2026 (885 households, 4,616 individuals; implying an average household size of roughly 5.2 persons) found 76.16% of the sample were currently displaced [288]. Among displaced households, 41.39% lived in apartments, 33.23% in formal collective shelters, 17.94% with host families, and 7.44% in tents or informal shelters [288]. In that assessment sample, 69.94% identified as Lebanese, 24.75% as Syrian, and 5.31% as other nationalities (predominantly Ethiopian and Sudanese) [288].

IOM/DTM and DTM Syria recorded significant cross-border flows into the Syrian Arab Republic, with 418,589 cumulative movements at four points of entry and 120,349 arrivals across 1,234 locations between 2 March and 8 June 2026 [309][303].

Of the 1.4 million people targeted in the Flash Appeal, UNFPA estimates approximately 390,000 are women of reproductive age, including an estimated 16,000 pregnant women, highlighting protection and health service needs among adult female cohorts [294]. Ministry of Public Health casualty data show children accounted for 8% of total casualties since 2 March (247 child deaths and 983 injured), while males constituted the majority of casualties (86%), indicating disproportionate mortality and injury by sex and notable child impact among conflict-affected cohorts [305].

Socio-Cultural

Socio-Cultural Overview

Displacement has heightened host community-displaced population tensions, with displaced groups perceived as affiliated with armed actors and sectarian polarisation rising amid renewed hostilities in early June 2026 [322][320]. More than one million people have been displaced since 2 March 2026, registered through government and humanitarian platforms [326][327]. Nearly 48,000 displaced migrants face language, status, and access barriers to assistance [341][345]. Relations between IDPs and host communities are mixed—some areas (Marj, Deir El Ahmar) report respectful relations, while parts of the Bekaa show heavy dependency of IDPs on hosts (Al Ain, Saaide, Bouday) and risk of social exhaustion if assistance is not conflict-sensitive [332]. Damage to cultural and religious heritage—particularly in Tyre and other southern sites—and evacuation orders affecting historic and religious neighbourhoods have deepened losses of identity and prompted appeals to UNESCO and national antiquities authorities [336][346][344]. Humanitarian and social-stability actors are implementing community dialogue, women- and youth-focused engagement, and Explosive Ordnance Risk Education to reduce tensions and build resilience, but funding and access constraints limit scale and sustainability [323][322].

Gender Dynamics

Displacement has disproportionately affected women and girls: since 2 March 2026, an estimated 620,000 women and girls have been displaced, with 85% living outside formal shelters, increasing protection risks [320][322]. As of 31 May, nearly 52% of people sheltered in collective sites were women and girls, with 130,369 individuals remaining displaced across 632 collective shelters, pressuring shelter capacity [345]. Overcrowded, privacy-lacking collective shelters heighten risks such as harassment, exploitation, intimate partner violence, unsafe access to WASH facilities, and psychosocial distress for women and girls [340][332]. Protection and justice services have been disrupted, making it harder for women and other at-risk populations to report violations or seek support [322]. Reproductive health needs are acute: an estimated 390,000 women of reproductive age, including some 16,000 pregnant women, are affected, and disruptions to maternal services have forced some women to give birth in unsafe conditions [340]. Women-headed households and other vulnerable groups—including persons with disabilities, pregnant and breastfeeding women, and widows—face elevated housing, food, and protection vulnerabilities requiring targeted support [331][332]. Gendered economic pressures are driving different coping patterns: men and boys face intense pressure to provide, debt accumulation, and risky coping, while women face reduced access to income-generating opportunities due to childcare, mobility, and safety constraints, as well as limited control over household resources [332]. UNHCR and protection partners report having reached more than 242,500 individuals with targeted protection interventions, including GBV prevention and response, while gaps in GBV and specialised support are widening due to resource and capacity constraints [331][323]. Humanitarian responses include GBV prevention and response, case management, multipurpose and regular cash assistance, mobile SRH and MHPSS services, and shelter upgrades and partitioning to improve privacy, but limited funding and operational constraints are reducing coverage and leaving significant needs unmet [331][340][347].

Impact of Socio-Cultural Drivers on Aid

Cultural norms and social hierarchies are shaping access to and acceptance of assistance in Lebanon. Displaced households disproportionately rely on negative financial coping mechanisms: 32.26% of households reported debt as a primary protection concern, and displaced households account for roughly 75% of debt cases, increasing eviction, legal, and psychosocial risks [332]. Many households also rely on host-family and displaced community solidarity networks for assistance, which affects targeting, acceptance, and the perceived legitimacy of formal aid [332][362]. SOGIESC populations and women-headed households face compounded barriers to services, as reported in Oxfam's digital needs assessment (April–May 2026) [332][324].

A GBV safety audit of 180 collective shelters found major protection gaps: 28% of shelters lack adequate space and 20% fail to separate female-headed households from unrelated men, with overcrowding, lack of locks, partitions, and lighting, and non-segregated or inaccessible WASH facilities heightening GBV risk [340]. Shelter monitoring shows concentrations of vulnerable groups in collective sites—approximately 6,800 older people (55+), 1,200 people with physical disabilities, 661 with mental disabilities, 386 with hearing impairments, and 392 with visual impairments—requiring inclusive, accessible, and culturally sensitive modalities and communications [362]. Access to culturally acceptable SRH and maternal care is further constrained by 182 verified attacks on healthcare (99 affecting medical transport and 61 affecting facilities), causing 125 deaths and 311 injuries [340]. Sustained pressure on host communities and the nearly 48,000 displaced migrants facing language barriers and restricted service access continue to undermine equitable aid reach, as noted in the situation report of 8 June 2026 [345][362]. Humanitarian coordination efforts are scaling GBV, PSEA, and survivor-centred services through partnerships with MoSA, UNHCR, and local actors, but entrenched social norms, protection gaps in shelters, healthcare attacks, and language barriers limit effective uptake and equitable distribution of assistance [340][362].

Environment

Environmental Conditions

Renewed hostilities since 2 March have caused widespread damage to homes, health facilities, roads, agricultural land and critical infrastructure, disrupting water and electricity services across Lebanon [440]. Hostilities have also sparked extensive fires, and independent reporting and government submissions note use of white phosphorus, raising risks of immediate fires, respiratory injuries and long-term soil and water contamination [445][448]. As of early June, nearly one million people remained displaced, placing significant pressure on local water and sanitation services [440]. Humanitarian operations continue to face constrained access — including cancelled or delayed missions, route restrictions, communications disruptions, GPS spoofing and unexploded ordnance contamination — which limit environmental assessments, rubble removal and the scale-up of WASH and contamination-mitigation activities [432][417].

Environmental Challenges

Use of white phosphorus and other munitions over populated areas has reportedly poisoned soil, water and air, causing long-term ecosystem damage and severe health effects including burns and respiratory injuries, with government figures linking more than 600 fires in southern Lebanon to white phosphorus use and increasing risks of soil degradation and contamination of agricultural land and groundwater [441][445][448]. Conflict-generated rubble and debris — Lebanon generated 14.5 million metric tonnes in 2024 — are compounding waste-management and environmental hazards and require structured rubble-recycling and environmental management plans to mitigate dust, hazardous contamination and polluted runoff [442][413]. Immediate public-health impacts include heightened risk of waterborne disease due to water infrastructure damage, contamination and service disruption, underscoring the need for monitored water quality, safe waste disposal and environmental risk assessments in recovery planning [445][417]. In response, WASH partners reached approximately 1,072,601 people with emergency water, sanitation and hygiene services as of 8 June [417][432], delivering 3,765,725 litres of potable water and 107,230 m³ to collective shelters via water trucking, supplying 851,000 litres of fuel to water establishments, and distributing 111,911 hygiene kits, 66,579 menstrual hygiene kits and 1,488 cleaning kits to affected populations [417].

Environmental Risks and Mitigation

A UNDP geo-rapid assessment in early June 2026 found extensive building damage and debris in Beirut and Mount Lebanon — 146 buildings completely destroyed, 264 partially damaged, 3,168 housing units destroyed and 4,437 damaged — producing an estimated 649,000 cubic metres of debris that poses immediate health, safety and environmental hazards for recovery operations [404]. Unexploded ordnance (UXO), explosive remnants of war (ERW) and unstable structures are impeding safe clearance and humanitarian access, increasing the need for specialized risk-management and safety protocols during debris removal [417][413]. International guidance recommends people-centred, circular rubble management to reduce environmental harm and speed reconstruction: prioritizing and pre-identifying removal and reuse sites, segregating hazardous materials (including asbestos and ERW), securing environmental permissions, and incorporating rubble management costs into response budgets [413][442][435]. UNOPS and related guidance further highlight rubble reuse as a mitigation measure that can cut environmental impacts, lower reconstruction costs and create local economic opportunities, provided adequate sorting and safety measures are in place [442]. Persistent funding shortfalls in the 2026 Flash Appeal — approximately 32% funded as of 11 June — and reduced in-kind contributions risk undermining sustained environmental mitigation actions, including safe debris management, restoration of waste and water systems and monitoring of contamination risks [417][432].

Conflict

Overview of the Ongoing Armed Conflict

Since 2 March 2026, hostilities between Israeli forces and Hezbollah have produced sustained airstrikes, drone and artillery strikes, and limited ground operations in southern Lebanon, with a notable escalation and geographic expansion of Israeli operations from 26 May 2026 onward [534][554][566]. Fighting has concentrated in southern governorates—particularly South and Nabatiyeh—and has extended beyond the Litani River into areas south of the Zahrani River, while Mount Lebanon and parts of Beirut's southern suburbs have also been affected [535][534]. By 1 June 2026, Israeli forces had reportedly occupied roughly 2,000 km² of southern Lebanese territory [554][534].

Lebanon's Ministry of Public Health (MoPHPHEOC) reported 15,194 total casualties (deaths and injuries) from 2 March to 11 June 2026, with the South (6,379), Nabatiyeh (6,333), and Mount Lebanon (1,182) the most affected; children comprised 8% of casualties (247 dead, 983 injured), and 94% of casualties were Lebanese (14,297) [535].

ACAPS estimated over 3,600 killed and nearly 11,100 injured since 2 March 2026 [534].

Humanitarian actors estimated large-scale displacement and severe protection needs. ACAPS estimated approximately 1.5 million people displaced nationwide by 7 June 2026 [534], while national DRM dashboards recorded sudden movements on 7 June 2026—approximately 4,000 families displaced in Nabatieh, 30,000 families in South governorate, and ~50,000 families (~200,000 people) from Beirut's southern suburbs—placing collective shelters at or beyond capacity [535]. A mobility assessment reported 135,669 people sheltering across 636 collective shelters as of 4 June, with thousands more in open spaces [521].

The conflict has significantly degraded critical services and infrastructure: strikes and access constraints have damaged health facilities and ambulances, reportedly killed and injured medical personnel, left some hospitals unable to accept patients due to capacity and financial/coordination gaps, and prompted WHO-supported nationwide trauma preparedness and shelter-based surveillance efforts [534][554][535]. Active hostilities, repeated strikes, and territorial occupation have constrained humanitarian access, forced temporary evacuation of collective shelters, and pushed many displaced people into open or tented sites with limited services, complicating relief operations [534][535][554].

Conflict Stakeholders

The principal actors are Hezbollah (a powerful Iran-aligned non-state armed group), the Lebanese state and its armed forces, and Israel as the external belligerent [580][547][591]. Hezbollah retains significant independent military capacity and political legitimacy, making disarmament contentious and heightening the risk that forced disarmament could trigger internal instability [547]. On 13 June 2026, Prime Minister Nawaf Salam publicly called for consolidation of state authority and the deployment of the Lebanese Armed Forces to conflict-affected areas to restore the state's monopoly on force [591]. The Lebanese Armed Forces have also sought to strengthen external military cooperation, including senior-level talks and visits with partners such as Pakistan, to bolster capacity amid the security crisis [596].

The United States reduced planned assistance to the Lebanese Army to \$36 million for fiscal year 2027, down from \$100–200 million annually in recent years, constraining a key state alternative to Hezbollah's military presence [580]. War-related losses and reconstruction needs are estimated in the range of approximately \$20–25 billion, with extensive infrastructure and property damage reported [590][582].

Peace Negotiations

Negotiations between the United States and Iran advanced toward a Pakistanbrokered memorandum of understanding that mediators said would be signed in mid-June and that reportedly includes Lebanon in its ceasefire framework [500][522]. Key mediators and regional actors engaged in the process included Pakistan, Qatar, Saudi Arabia, and Turkey, while the United States led talks with Iran and hosted parallel Lebanon–Israel negotiations in Washington [522][500][481]. The United States pressed to keep talks on track and publicly urged Israel to curb strikes in Lebanon to avoid undermining the diplomatic process [606][504].

Despite this diplomatic momentum, Israeli strikes and repeated airspace incursions in southern Lebanon continued. Lebanese authorities reported 3,491 Israeli ceasefire violations (airspace incursions, strikes, and ground incidents) since 17 April [482], and Lebanese officials reported 47 Lebanese military and security personnel killed since 2 March [482]. These ongoing violations have weakened adherence to understandings reached since April [493][482][554].

The main obstacles to a durable ceasefire are Hezbollah's rejection of the arrangements negotiated directly between the Lebanese Government and Israel, Israel's reluctance to formally link the Lebanon file to broader US–Iran talks, and ongoing onground violations that reduced incentives for a sustained ceasefire after the early June clashes [529][534][532]. Analysts and briefs warned that any durable ceasefire in Lebanon is operationally linked to progress in US–Iran negotiations, narrowing the diplomatic window after the 7 June escalation [534][529].

An estimated 620,000 women and girls have been displaced since 2 March, with 85% living outside formal shelters and facing heightened protection risks including exploitation and genderbased violence, increasing protection risks and complicating recovery and negotiations [532][529].

Conflict Outlook

The 7 June 2026 Israeli strikes on Beirut's southern suburbs (Dahiyeh) triggered Iran to launch ballistic missiles at Israel in direct response, illustrating how Lebanese operations now generate wider regional escalation [532][534][519]. Parallel US–Iran military exchanges and intense diplomatic mediation through Pakistani and Qatari channels left talks reportedly close to a framework agreement but fragile amid strikes and reprisals in early-to-mid June [550][486][477][542].

Hezbollah's rejection of Lebanon-specific ceasefire arrangements and Israel's reluctance to link the Lebanon file to US–Iran negotiations significantly reduce the likelihood of a durable ceasefire in the near term and have narrowed the diplomatic window following the 7 June escalation [534][529]. Progress toward a US–Iran agreement could create incentives for deescalation, but recent crossborder strikes and Israeli actions in Beirut risk derailing that prospect, making a negotiated settlement possible but fragile and contingent on sustained external mediation and restraint [486][504][616][534].

Continued hostilities, the operational linkage between the Lebanon front and US–Iran negotiations, and the risk of tit-for-tat strikes make further largescale displacement and protection deterioration likely in the coming weeks [534][529][521]. Humanitarian conditions are already severely strained, with thousands sheltering in open spaces and collective shelter systems overstretched, compounding the humanitarian toll of any further escalation [521].

Displacement

Displacement Overview

As of early June 2026, nearly one million people remained internally displaced in Lebanon, with the vast majority hosted outside collective shelters [624][677]. Official counts show 130,369 individuals in 632 collective shelters as of 31 May 2026, rising to 135,669 individuals in 636 collective shelters as of 4 June 2026, reflecting recent increases driven by renewed displacement orders [662][621]. Renewed displacement orders in late May and new orders in early June produced sharp spikes in movement: orders around 27 May and further population movements on 7 June led authorities to report an estimated 4,000 families displaced in Nabatieh and 30,000 families in South governorate, alongside preliminary estimates of approximately 50,000 families displaced from Beirut's southern suburbs on that day [652][628]. Shelter capacity has been overwhelmed in Saida and Tyre (South Governorate), driving newly displaced families toward North Governorate and parts of Bekaa where additional shelters and tented sites have been activated [662][677][669]. Women and girls represent around 52% of those in collective shelters, and nearly 48,000 displaced migrants in Lebanon face specific vulnerabilities and barriers to services [662][619]. Cross-border flows into the Syrian Arab Republic remained substantial: IOM/DTM recorded 418,589 movements into Syria at tracked points of entry as of 8 June 2026, with Syrian nationals comprising about 97% of recorded movements and approximately 32,738 people returning to Lebanon since the 27 April peak [650]. Displacement patterns are highly fluid, with repeated returns and secondary displacement complicating registration and producing variability between institutional estimates [652]. IOM, UN agencies, and national authorities are scaling Displacement Tracking Matrix (DTM) monitoring and coordinating under the revised Flash Appeal to track trends, inform prioritization, and adapt the multi-sectoral response as the situation remains protracted and returns largely temporary and unsustainable [624][638][652].

Living Conditions of the Displaced Population

The vast majority of nearly one million internally displaced people in Lebanon are hosted in informal arrangements outside formal collective sites, placing sustained pressure on communities and services [624][619]. Collective shelter capacity is critically stretched, with sites in Beirut, Tyre, and Saida reported at or near full capacity as of early June 2026 [621][628]. Children, older people, persons with disabilities, and people with chronic conditions face additional barriers to services; the Shelter Monitoring Dashboard recorded roughly 6,800 older people, 1,200 persons with physical disabilities, 661 persons with mental disabilities, 386 persons with hearing impairments, and 392 persons with visual impairments in collective shelters [628]. An estimated 620,000 women and girls have been displaced since 2 March 2026, with approximately 85% living outside formal shelters, heightening protection and GBV risks [628][635]. Non-Lebanese displaced people — Syrians, Palestinians, and migrants — face acute barriers to formal accommodation and access to services, as well as increased legal insecurity linked to loss of documentation, while UNHCR continues targeted support for birth registration and residency assistance [652][618].

Access to health care is constrained by facility damage, workforce shortages, and attacks on health care — WHO has recorded 202 attacks since 2 March — and the Ministry of Health reported 3,526 deaths and 10,733 injuries since 2 March (data as of 9 June 2026), undermining continuity of emergency, trauma, MHPSS, and SRH services [630]. SRH needs are acute, with UNFPA estimating large numbers of women of reproductive age and pregnant women among the displaced and emphasizing urgent gaps in maternal services, safe deliveries, and SRH supply provision [667]. WASH and sanitation conditions are poor in many displacement settings, with a digital needs assessment finding 56.93% of latrines non-functional and pervasive concerns about cleanliness, privacy, and safety in shared facilities [668]. Food and nutrition support has been scaled but remains insufficient: partners report distributions totalling approximately 12.6 million meals and 141,192 ready-to-eat rations since 2 March, yet around 80% of collective shelters are providing only one hot meal per day and minimum food security needs are unmet in many sites [632].

Psychosocial distress and protection needs are widespread; protection partners and UNHCR have reached hundreds of thousands with interventions, but gaps remain for GBV, MHPSS, and specialized child and disability services, and UNFPA reports urgent needs for dignity kits and expanded safe-space and mobile GBV services [618][667][668]. Cash assistance is being scaled — MoSA shared beneficiary lists covering roughly 100,000 households for E-MPCA Batch C, and UNHCR/WFP reached over 137,000 refugee households with EMPCA in May — while significant funding shortfalls persist for the emergency response, constraining longer-term income support and shelter solutions [632][618][628]. Humanitarian operations are further constrained by security, access, and logistical challenges, with 203 HNS-supported humanitarian movements recorded between 2 March and 11 June and multiple cancellations, limiting delivery and scale-up of multi-sector assistance and reinforcing the need for urgent, flexible funding to sustain life-saving services [669][628].

Drivers of Displacement

The primary driver of displacement is renewed hostilities since 2 March 2026 — including Israeli attacks and evacuation orders — which have forced more than one million people to flee their homes [668][675][661]. Widespread destruction of homes, hospitals, schools, and other critical infrastructure has reduced access to basic services, with about 17 schools destroyed and roughly 500,000 children out of classrooms, further impeding return and recovery [675]. Humanitarian assessments identified livelihoods and food as the top needs of displaced and host households, and reported heightened protection risks including increased exposure to harassment, exploitation, gender-based violence, and psychosocial distress among displaced populations [668][631].

Pre-existing socioeconomic collapse and loss of livelihoods have compounded these displacement drivers, with sharp price shocks — bread prices up approximately 15% and fuel prices up approximately 84% between mid-February and mid-May 2026 — contributing to projected acute food insecurity affecting about 1.24 million people (IPC, 29 April 2026) [652][643]. Regional spillover and deteriorating security in host countries have also contributed to return movements from Lebanon and Iran to Syria and Afghanistan, often under adverse conditions [641][626]. Secondary displacement is a persistent dynamic, as returnees frequently face deteriorating conditions in areas of origin — including ongoing hostilities, damaged housing, and lack of services — leading to repeated and largely unsustainable returns [652].

Integration

Large-scale, prolonged displacement and heavy reliance on informal hosting arrangements are constraining integration prospects and placing sustained pressure on host services and communities in Lebanon [619][632]. Social cohesion is under strain, with reports documenting rising host community tensions, perceptions of displaced populations' affiliations increasing mistrust, and risks of host-community exhaustion; social-stability partners have delivered community dialogue, explosive ordnance risk education, and other engagement activities to mitigate these tensions [652][668][635][632].

Education access is severely disrupted, with 446 schools non-operational as of 28 May, though Education Sector partners reached 262,384 displaced children through formal modalities (94,073 in-person; 163,420 online) and temporary learning hubs have been established to mitigate loss of schooling [652][632][669]. Health and social services are overstretched: WHO reported widening gaps, medicine and workforce shortages, and an estimated 13,500 displaced pregnant women in shelters, while IOM and partners have supported access to essential health services for tens of thousands (IOM: 18,394) and WHO has delivered trauma and surgical kits to priority hospitals [630][619]. Protection and MHPSS capacity has expanded but remains insufficient — UNHCR and partners provided MHPSS to nearly 8,700 people and IOM to 4,301 since 2 March, alongside national MHPSS outreach, trainings, and referrals — yet gaps in GBV and specialized support are widening [618][619][644].

Displaced households face mounting financial pressures from rising rents, limited income opportunities, and depleted savings, undermining access to durable livelihoods [618][627]. Humanitarian livelihood programming includes IOM-led emergency cash-for-work targeting 14,400 individuals and other livelihood activities within the Flash Appeal [624][627]. Government and institutional responses show active engagement: the Ministry of Social Affairs shared beneficiary lists for emergency MPCA covering approximately 100,000 households and is coordinating with humanitarian actors; IOM and MoSA are co-leading Site Management and Coordination; and partners are working to harmonize targeting and transition emergency cash into regular MPCA from June, while UNHCR is implementing cash-for-shelter, shelter rehabilitation, and tenure-support measures to sustain longer-term settlement options [632][619][618][624][622][627].

Humanitarian Access

Humanitarian Access Overview

Humanitarian access in Lebanon remained highly constrained and increasingly reliant on notification mechanisms despite the declared ceasefire, with movements to areas south of the Litani River and parts of Nabatieh continuing amid heightened insecurity and operational unpredictability [702][695]. With support from the Humanitarian Notification System (HNS), partners facilitated 184 humanitarian movements between 2 March and 8 June and 203 movements by 11 June, the majority south of the Litani, coordinated with UNIFIL and civil-military notification arrangements [702][695]. Operational challenges—including delays, restricted operational windows, route constraints, communications disruptions (including GPS spoofing), unexploded ordnance (UXO) contamination, and cancellations—frequently disrupted missions; between 26 May and 9 June, 22 movements (primarily to Tyre District) were cancelled and one postponed [695]. Primary entry points for humanitarian cargo remained the Port of Beirut and the Al Masnaa border crossing, while domestic convoy movements to hard-to-reach southern locations were coordinated with WFP and the Logistics Cluster—planned convoys included Sarba (9 June), Rmeish (11 June) and Borj el Moulouk (12 June) in Nabatieh governorate [702]. Coordination bodies including the Inter-Sector Coordination Group (ISCG), Operational Coordination Groups (OCGs), the Ministry of Social Affairs and the Disaster Risk Management unit continued to monitor displacement and harmonize targeting and operational modalities to inform and facilitate humanitarian movements under these constraints [702][695].

Barriers to Humanitarian Access

Multiple overlapping barriers continue to severely restrict humanitarian operations across Lebanon. Key physical and logistics obstacles include destroyed bridges—notably over the Litani River—that have severed connectivity between Sour (Tyre) and Nabatiye and isolated over 150,000 people, while hard-to-reach areas, UXO contamination, threats of additional infrastructure destruction, growing domestic route congestion, and supply-chain strain compound operational difficulties [704][702]. Sustained attacks on health care have sharply reduced service availability: since 2 March, 196 attacks have been recorded (131 deaths, 379 injuries), including 50 attacks since the 17 April ceasefire and a 1 June attack near Jabal Amel Hospital that injured 39 health workers; as a result, 42 primary health care centres and 3 hospitals have been closed and 17 hospitals damaged [737][703]. Affordability barriers, medicine shortages, workforce shortages and reduced specialized services further limit access to care, and the Health Sector estimates an urgent need of USD 61.5 million under the Revised Flash Appeal with only USD 12.8 million currently available—representing approximately 17% of revised Flash Appeal funding (June–August 2026) [703][702]. Displacement and shelter constraints also impede assistance: nearly one million people are internally displaced, the vast majority hosted informally outside collective sites, while roughly 135,000 remain in collective shelters where overcrowding, poor sanitation, lack of privacy and dependence on host families elevate protection risks [699][714][693]. Negative coping strategies were reported by 32.26% of households, and protection emerged as a major need among displaced respondents (16.38%) in assessments [699][714][693]. Shelter populations include significant numbers of people with specific vulnerabilities: approximately 6,800 older people (55+), 1,200 persons with physical disabilities, 661 persons with mental disabilities, 386 persons with hearing impairments, and 392 persons with visual impairments in collective shelters [700]. Non-Lebanese displaced populations and an estimated 48,000 displaced migrants face additional legal and administrative barriers—including movement restrictions, risk of arrest, repeated displacement, and government restrictions on access to shelter and basic services—that further limit their ability to access assistance [700][704][730].

Political and social obstacles include perceptions of displaced people's affiliation with armed groups and intensified competition over scarce resources, which have fuelled sectarian polarisation, social tension, discrimination, and localized harassment or attacks against displaced persons [704].

Administrative bottlenecks and an absence of state-led displacement data produce information gaps and delays, making targeting and timely scale-up dependent on humanitarian assessments [704]. Operational constraints are further compounded by sharply reduced in-kind donation volumes (reported at approximately 18% of previous levels) and wide funding shortfalls across appeals, including IOM's revised request of USD 42.37 million and UNFPA's appeal of USD 25 million with approximately 16% funded, collectively undermining the capacity to expand and sustain assistance at scale [702][697][730][728].

Humanitarian Access Gaps

Ongoing hostilities have produced large-scale internal displacement and severely strained humanitarian access across Lebanon. Nearly one million people are internally displaced, with IOM estimating over 48,000 displaced migrants who face specific barriers to assistance and often avoid government-managed collective sites [730][699][712]. As of 4 June 2026, 135,669 people remained displaced across 636 crowded collective shelters, with thousands more in informal arrangements [718]. Displacement is concentrated from southern areas toward Beirut, Mount Lebanon and northern governorates (Akkar, Baalbek-Hermel), and returns are largely temporary and unsustainable, undermining access planning [704]. South and Nabatiye governorates face worsening food insecurity at IPC Phase 3 through at least September 2026, driven by market collapse and constrained humanitarian access [704]. Humanitarian delivery is impeded by limited access to healthcare, shelter and WASH services, overcrowding in hosting areas, and stretched service points and response capacities [699][706]. Migrants face language, information and status-related barriers that reduce uptake of assistance, while women and girls face heightened protection risks including gender-based violence (GBV), and children are reporting elevated psychosocial distress [730][704][706]. Key sectors with critical shortfalls include health, shelter, WASH, food security, protection (including GBV response), mental health and psychosocial support (MHPSS), livelihoods and humanitarian border management [730][699]. Most displaced people rely on informal hosting outside formal sites, limiting humanitarian reach and monitoring. In response, humanitarian actors are calling for expanded multi-sectoral assistance, strengthened mobility and early-warning data, and targeted site-management support; IOM is supporting NGOs on site management for migrants, and Oxfam's April–May needs assessment is being used to refine targeting and programme design [699][730][693][706][707].

Information, Communication and AAP

Information Landscape

Information on the Lebanon escalation is compiled from coordinated inter-agency and government sources (OCHA/ISCG flash updates and operational coordination groups), national authorities (MoSA, DRM, MoPH), UN agencies (UNHCR, IOM, UNFPA), international NGOs (IRC), and community-facing digital assessments, which are used together to inform response planning and monitoring [781][765][786][797][806][774]. Key operational datasets and tools in use include the Emergency Rapid Needs Assessment (ERNA), real-time digital dashboards tracking displacement, aid distribution and in-kind donation flows, site-management/occupancy trackers, and complaint-and-feedback monitoring systems used by site-management actors [781][765][797]. IRC reported more than 1 million people displaced as of 8 June 2026 [806], while OCHA/ISCG situational reporting described nearly one million internally displaced people and approximately 48,000 displaced migrants in early June, with 134,800 people registered in 639 collective shelters as of 8 June 2026 [777][765]. WASH sector partners reported reaching 1,074,702 people as of 11 June and delivering 851,000 litres of fuel since 2 March to sustain public water services for an estimated 841,128 people [781]. Coordination and data-sharing practices have been intensified since the revised Flash Appeal (launched 5 June), with ISCG/OCGs working with MoSA and DRM to harmonize targeting and operational modalities, and sectors reviewing indicators to align with SMART criteria to strengthen response monitoring [781][765]. UNFPA and partners plan deployments of rapid needs-assessment teams and intend to boost national and UN information-management capacity to support continuous field assessments and sector coordination [773]. Data reliability remains constrained by access and political limitations that can filter institutional reporting, while open-source and community reporting have expanded coverage but can diverge from institutional datasets; humanitarian actors are calling for strengthened mobility and displacement monitoring and early-warning systems to improve timeliness and validation [793][777].

Communication Channels

Oxfam's Digital Needs Assessment (April–May 2026) found 78.98% of surveyed households preferred WhatsApp as their feedback mechanism, with face-to-face reporting second at 11.98% [764]. Oxfam recommends prioritising accessible, secure WhatsApp-based feedback channels while retaining limited in-person options for people without reliable digital access or privacy [764]. OCHA Flash Updates noted that humanitarian access is increasingly reliant on notification mechanisms; the Humanitarian Notification System (HNS) supported 203 humanitarian movements between 2 March and 11 June, though missions frequently faced delays, communications disruptions, and GPS spoofing [781]. OCHA-coordinated logistics movements included convoys to Sarba (9 June), Rmeish (11 June), and Borj el Moulouk (12 June) to deliver food and life-saving assistance in Nabatieh governorate [765]. WHO and the Lebanon Health Sector called for expanded risk communication and community engagement, and reported support for radio/CBRN preparedness alongside urgent needs to bolster surveillance and risk communication capacities [795][785]. Displaced residents are also using commercial satellite imagery to check on home damage where physical access remains restricted [790].

Barriers to Information

Multiple overlapping barriers limit information access for affected populations in Lebanon. According to Oxfam's Digital Needs Assessment (April–May 2026), while WhatsApp is the preferred feedback channel, many women lack personal phones, share devices, or have no privacy to report sensitive issues; migrant women face language barriers; and older persons and persons with disabilities often prefer verbal or face-to-face channels [764]. The assessment also notes selection bias: households with limited phone or internet access, low digital literacy, or unstable communication channels were less likely to participate, excluding some of the most vulnerable from digital information and feedback systems [764].

Poor internet connectivity, lack of devices, and limited learning spaces further restrict digital information access and remote outreach for displaced students and remote learners [766][765]. According to IOM, over 48,000 displaced migrants in Lebanon face language constraints, limited information on services, and fears linked to migration status that reduce their access to information and assistance [797]. Infrastructure and operational disruptions — including repeated power outages, burning of power poles, road closures, communications disruptions, GPS spoofing, unexploded ordnance contamination, and cancelled or delayed humanitarian movements — have degraded communication channels and constrained information flow to affected communities [788][781]. Journalists have been killed, displaced, or cut off from information sources, and misinformation risks have risen, narrowing the public information space and limiting independent reporting [804]. Low complaint reporting and limited awareness of how to file grievances reflect lack of trust, fear of consequences, and short engagement periods, undermining accountability and two-way information channels [764].

Misinformation and Rumors

Social media and peer comments are major vectors of harmful information that can reshape perceptions of health care, with a small number of highly active or inauthentic accounts able to dominate discussions and influence wider public sentiment [793]. Many people in the region rely heavily on social networking sites for news, a consumption pattern that algorithms and viral content can amplify into sensational or misleading narratives [813]. Harmful information has eroded trust in health services and can discourage care-seeking, prompting the Health Sector and the Ministry of Public Health to call for expanded risk communication, community engagement, and public information to restore confidence and encourage early care-seeking [793][785]. Operational efforts to counter misinformation include MoPH hotlines and helplines, sector-led public information dissemination (24 public information activities recorded), and UNHCR and partners' outreach reaching tens of thousands through information sessions and awareness activities to provide verified guidance and protection messaging [791][765][769]. Health workers and community leaders — identified as influential, trusted voices — are being engaged in outreach and GBV/SRH awareness campaigns, while MHPSS actors are using social media posts and digital tools to share verified psychosocial support and referral information [773][792].

Accountability Needs and Transparency

Inter-agency coordination has prioritized strengthening response monitoring through real-time digital dashboards tracking displacement, aid distribution, and in-kind donation flows to improve transparency and accountability [765]. The Humanitarian Notification System supported 203 humanitarian movements between 2 March and 11 June, facilitating delivery of life-saving assistance but remaining constrained by insecurity and operational unpredictability [781]. Awareness of complaint mechanisms remains critically low: according to Oxfam's Digital Needs Assessment, 69.3% of surveyed households did not know how to file a complaint, with awareness lowest among returned households (22.43% aware) and displaced households (31.26% aware) [764]. Feedback channel preferences show 78.98% of households prefer WhatsApp (59.89% of displaced households), 11.98% prefer face-to-face, and SMS and other channels account for under 9%, indicating a strong need to prioritize secure WhatsApp channels while retaining limited in-person options [764]. UNFPA is leading enhancements to collective PSEA and accountability to affected people (AAP) as host agency for the Inter-Agency PSEA Network and the AAP Working Group [773]. The Government of Lebanon relief platform demonstrates large-scale digital registration potential, with over 1 million people self-registered as displaced as of 1 April, which could be leveraged for information sharing and feedback mechanisms [782]. Key gaps undermining accountability include widespread lack of awareness on complaint channels, constrained access due to insecurity, and severe funding shortfalls that limit partners' capacity to maintain monitoring and AAP systems — the initial 2026 Flash Appeal achieved approximately 60.3% coverage (March–May), and the revised appeal left a funding gap of approximately USD 454 million at extension outset [764][781][765][766]. The Health Sector has specifically called for expanded risk communication and community engagement to improve awareness of services and referrals, reinforcing the need for investment in accessible, context-appropriate information and feedback channels [785].

Legal and Policy

Legal and Policy Framework

The Government of Lebanon is leading the national humanitarian response under the Lebanon Response Plan (LRP), with the Ministry of Social Affairs (MoSA) as the focal point and close coordination with UN agencies (OCHA, UNDP, UNHCR) and humanitarian partners. UNHCR co-leads key sectors including Protection and Shelter and co-chairs cash coordination mechanisms alongside MoSA and NGOs [861][869][886]. IOM, working with MoSA, the Ministry of Public Health, and the Ministry of Interior, supports site management, displacement tracking (DTM), and sector coordination to align humanitarian interventions with national authorities [876][852]. International humanitarian law protections for health care — including Geneva Conventions principles and UNSCR 2286 — are being referenced amid repeated attacks on medical facilities and personnel [894]. Calls for accountability and strengthened legal recourse have been made by rights organisations, including recommendations that Lebanon ratify the Rome Statute and adopt domestic legislation criminalising war crimes and crimes against humanity to enable investigations and prosecutions [860][878]. The UN human rights office (OHCHR) announced deployment of an independent assessment mission to Lebanon in mid-June 2026 to document alleged violations of international humanitarian law and international human rights law by all parties since 2 March, with findings expected at the end of July 2026 [875][882][897][879]. Ceasefire and mediation efforts have proceeded through negotiated, external mediation including a US-brokered ceasefire agreement in mid-April, but rejection of some government-brokered arrangements by armed actors and the linkage of the Lebanon file to wider US–Iran negotiations have undermined prospects for a durable ceasefire and complicated legal-political pathways to conflict resolution [858][853].

Humanitarian Impact of Legislation

Lebanese authorities have restricted access to shelter and basic services for non-Lebanese populations, including displaced Syrians, Palestinian refugees, and migrant workers [858]. Non-Lebanese displaced people report movement restrictions, risk of arrest, and repeated displacement that further limit access to services and livelihoods [863][858]. IOM estimates nearly 48,000 displaced migrants face language barriers and restricted service access, with many unable or unwilling to use government-managed collective sites, increasing their vulnerability [855][876]. Many displaced and returnee populations also face lost, damaged, or expired civil documents, creating legal insecurity and limiting access to public services and assistance [884][873]. In May 2026, UNHCR assisted 221 refugee children with birth registration and 144 refugees to access legal residency, reflecting ongoing documentation support needs [869]. An estimated 620,000 displaced women and girls — 85% of whom live outside formal shelters — face heightened protection risks, while disrupted justice and support services hinder reporting and assistance [853]. UNHCR and protection partners have reached over 242,500 people with targeted protection interventions since the escalation, but the revised Flash Appeal remained severely underfunded at US\$206.2 million (32.2% of the US\$639.9 million requested as of 11 June), constraining the scale and continuity of legal assistance and protection programming [869][856]. The WHO Surveillance System for Attacks on Health Care recorded 203 attacks in Lebanon resulting in 135 deaths and 394 injuries among on-duty healthcare workers as of 11 June 2026, with multiple hospitals and primary health centres closed, indicating breaches of IHL and sharply reduced health system capacity [856][903]. Repeated strikes damaging hospitals, ambulances, and health personnel have delayed emergency response times and constrained aid delivery, undermining protection for patients and humanitarian workers [858][903]. Increased threats to peacekeepers and humanitarian staff — including multiple UNIFIL peacekeeper fatalities between 29 March and 3 June — further exacerbate operational risks for aid delivery [858].

International Agreements Impacting the Crisis

Multiple US-brokered ceasefire arrangements have been announced since 16 April 2026, but implementation has been inconsistent. Hezbollah rejected key arrangements reached through direct Lebanese government–Israel negotiations, reducing prospects for a durable ceasefire [875][862][853][858]. The 7 June escalation — including Iran's ballistic-missile response to strikes in Lebanon — demonstrates that the Lebanon front is linked to broader US–Iran negotiations, a dynamic that has narrowed diplomatic space for stabilizing agreements [853][858]. International actors and regional groups have called for respect for international law and implementation of relevant UNSC resolutions, including calls to implement UNSC Resolution 2803, and policy proposals have urged stronger measures to address impunity for violations of IHL, but operationalizing these demands has been limited to date [917][907]. UN peacekeeping assets are operational but under strain: three UNIFIL peacekeepers were killed on 29–30 March and a fourth on 3 June, and humanitarian movements increasingly rely on coordination with UNIFIL and the Humanitarian Notification System (HNS) to access southern areas [858][864]. Between 2 March and 11 June 2026, 203 HNS-supported humanitarian movements were facilitated, including 170 missions south of the Litani River, but operations faced cancellations, delays, route constraints, communications disruptions, and UXO contamination — including 22 missions to Tyre District cancelled between 26 May and 9 June due to security risks [864]. Humanitarian coordination is led under the Lebanon Response Plan, with MoSA, UNHCR, OCHA, and UNDP co-leading inter-sector groups and protection/shelter clusters [869][861]. The initial 2026 Flash Appeal (March–May) reached 60.3% coverage (USD 185.9 million of USD 308.3 million requested), with in-kind donations at just 18% of prior volumes, and the revised appeal (June–August) raised total requirements to USD 639.9 million with a major funding gap remaining [852][858]. Despite government engagement with UN coordination mechanisms, the combination of constrained access, funding gaps, and regional linkage to US–Iran talks has limited the effective implementation of ceasefire arrangements and other international measures addressing the crisis [875][869][864].

Limitations of Current Policies

The Government's inability to fully manage internal displacement and administrative bottlenecks mean displacement data is largely produced through humanitarian assessments rather than state-led systems, creating persistent information gaps that delay and complicate response planning [858]. Widespread infrastructure damage — including destruction of key bridges over the Litani River that have severed connectivity and isolated over 150,000 people — has compounded access constraints for civilians and responders [858]. Protection and GBV responses are insufficiently covered by current policies and capacities, particularly given the scale of displacement and the concentration of vulnerable populations outside formal shelters [853]. Accountability and legal gaps persist at the policy level: Lebanon has yet to ratify the Rome Statute or enact domestic legislation criminalizing war crimes and crimes against humanity, limiting avenues for investigation and prosecution of alleged violations documented since 2 March 2026 [860][875]. Health-care protection and service provision are critically weakened: WHO and health-sector partners recorded 196 attacks on health services since 2 March 2026, resulting in 131 deaths and 379 injuries, with many facilities operating at reduced capacity and limiting access to essential care [880]. Severe funding shortfalls continue to erode response capacity across all sectors, with the initial Flash Appeal achieving only 60.3% coverage and the revised appeal facing a substantial gap against USD 639.9 million in requirements [852][858]. Together, these structural, legal, and operational gaps underscore the need for policy reforms including domestic accountability legislation, improved civil documentation systems, dedicated protection frameworks for marginalized displaced populations, and sustained international funding commitments.

Food Security and Livelihoods

Access to Food and Market Functionality

Food security in Lebanon is worsening, with acute food insecurity projected to affect approximately 1.24 million people (up from 874,000 before the March escalation), driven by displacement, income loss, and rising living costs [939]. By mid-May 2026, overall food prices were 10–20% above 2025 levels, bread prices rose about 15% after the March escalation, and fuel prices increased roughly 84% between mid-February and mid-May 2026, amplifying transport and distribution costs [939]. Market functionality has collapsed or is severely constrained in southern areas, with crisis (IPC Phase 3) levels expected across South and Nabatiye governorates through September 2026 due to sustained attacks, collapsed markets, and constrained humanitarian access [939]. Attacks and infrastructure damage — including the 8 April strike on the Qasmieh Bridge, which damaged the last operational bridge linking areas south of the Litani River — have disrupted supply chains, damaged roads and bridges, and isolated more than 150,000 people, limiting commercial and humanitarian access [939]. While most markets across areas of operation continue to function, affordability and eroding purchasing power are increasingly undermining households' ability to meet needs and to fully use cash assistance, and movement restrictions in the Israeli-declared military zone affect an estimated 28,500 people [951][939]. Local food availability is further threatened by weakened agriculture — more than 22% of agricultural land damaged and 76% of farmers in southern Lebanon affected — combined with prior drought (2025 rains approximately 50% below average) and a likely 10% below-average rainfall forecast for June–August 2026 [939].

Coping Mechanisms

Harmful coping strategies are increasing among displaced and non-Lebanese households. According to Oxfam's Digital Needs Assessment (April–May 2026), 32.26% of households reported using debt as a financial coping mechanism, with displaced households accounting for roughly 75% of these cases; rent-related debt is driving eviction risks and psychosocial distress [928]. UNHCR and IOM reporting indicate negative coping strategies include reduced household spending, child labour, early and forced marriage, and transactional or survival sex, as shelter and accommodation options shrink [939][940]. Food consumption has been widely reduced in collective sites, with around 80% of shelters providing only one hot meal per day [944][951]. Many returnees from Lebanon lack immediate work opportunities, and households are increasingly relying on savings, remittances, loans, and external assistance to bridge income gaps, with casual daily labour being the most common income source where available [962][979]. To mitigate these negative coping strategies, UNHCR and cash-sector partners have scaled emergency cash responses, including emergency protection cash and multipurpose cash assistance (e.g., 13,550 EPCA recipients and integration of refugee households into MPCA/EMPCA reaching tens to hundreds of thousands), though needs remain unmet [940].

Livelihood Disruption

Livelihoods across Lebanon have been severely disrupted by the combination of conflict, displacement, and economic shocks. Oxfam's April 2026 digital needs assessment found livelihoods to be the top priority among respondents (47.12%), with displaced households disproportionately affected [928]. Price shocks — including food prices rising 10–20% by mid-May 2026 versus 2025 and fuel prices increasing approximately 84% between mid-February and mid-May 2026 — have significantly eroded household purchasing power, raised transport and production costs, and reduced employment prospects [939][956].

Key economic sectors have been hard hit. Tourism and related services have contracted sharply, with airport arrivals down 40.2% and total passenger traffic down 34.2% in the first five months of 2026, reducing incomes for hotels, restaurants, transport, and service workers [956]. The industrial sector is reported to be contracting by 45–50%, with factory shutdowns, rising closures, and loss of daily and seasonal jobs as employers struggle to pay salaries [963]. Agriculture, construction, and manufacturing — sectors with limited flexibility — have been hit by damage to agricultural land and evacuation orders affecting 76% of farmers in southern Lebanon, further undermining local income sources [939][956].

In response, humanitarian actors are scaling cash-based livelihood support. The Cash Working Group mobilized MPCA targeting and disbursements starting 9 June, and IOM's Flash Appeal included emergency cash-for-work livelihoods activities targeting 14,400 individuals with USD 1,000,000 to provide short-term income [951][954].

Impact on Agricultural Activities and Production

Lebanon's agricultural sector has been severely affected by compounding shocks since 2019. By 27 March, more than 22% of agricultural land had been damaged and Israeli evacuation orders affected 76% of farmers in southern Lebanon, directly reducing local production capacity [939]. The escalation has driven large-scale displacement of over 1 million people nationally, which, together with evacuation orders, has removed labour from farming communities and risks longer-term loss of agricultural activity in affected southern areas [930][939]. Agriculture, together with construction and manufacturing, accounted for approximately 16% of GDP in 2024, amplifying the broader economic consequences of agricultural disruption [956][967].

Infrastructure destruction has compounded these impacts: attacks have destroyed water systems, roads, bridges, and transport routes, disrupting supply chains and isolating conflict-affected communities [939]. Rising input and transport costs — with fuel prices up approximately 84% between mid-February and mid-May 2026 — are increasing agricultural production costs and further reducing the purchasing power of displaced and refugee households [939].

Climatic conditions add further pressure. Lebanon experienced drought in 2025 with rains approximately 50% below average that damaged crops; forecasts indicate a likely 10% below-average rainfall for June–August 2026, and expected El Niño conditions over 2026–2027 could aggravate medium-term agricultural impacts [939].

Food and Livelihood Needs and Current Response

Since 2 March, Food Security and Agriculture partners have distributed over 13.3 million hot and cold meals, approximately 145,436 ready-to-eat (RTE) rations, and roughly 170,286 bread bundles, alongside food parcels and core relief items to displaced and affected households [951][940]. As of 8 June, 134,800 people (35,600 families) were registered in 639 collective shelters; due to resource constraints, approximately 80% of shelters are providing only one hot meal per day rather than the recommended two, and partners report that minimum food security requirements are not being met across the majority of sites [927][944]. An RTE pipeline shortfall is projected from mid-June, with estimated gaps of approximately 700 RTEs in the North and 384 in Akkar [927][944][951].

Cash modalities have been scaled up significantly. UNHCR and WFP reached over 137,000 vulnerable refugees with emergency multi-purpose cash assistance by May, and the Ministry of Social Affairs shared beneficiary lists covering approximately 100,000 households for Batch C of a blanket E-MPCA [940][944][954]. The Cash Working Group scheduled targeting and disbursements to commence from 9 June, while IOM's appeal includes MPCA for 49,500 individuals (USD 6.0 million) and livelihoods activities targeting 14,400 people (USD 1.0 million) [954]. A needs assessment by Oxfam reports displaced households prioritize food baskets (49.5%) and RTE kits (9.22%), and identifies livelihoods and cash as the primary need to cover rent, fuel, infant items, and other essentials, with disproportionate impacts on women-headed households [928].

Coordination mechanisms are active: ISCG and Operational Coordination Groups work closely with MoSA and the Disaster Risk Management unit, real-time digital dashboards track displacement and aid flows, and sector working groups – including the Cash Working Group, UNHCR, and IOM – are harmonizing targeting and operational modalities with government counterparts [951][927][944][932][954]. Critical funding gaps persist: the revised Flash Appeal for March–May 2026 was only 60.3% funded (USD 185.9 million of USD 308.3 million), and in-kind donations are at approximately 18% of prior levels, underscoring that scale-up of both cash and in-kind assistance remains urgently required [927][944][951].

Education

Learning Environment and Facility Conditions

As of 11 June, 446 of Lebanon's 3,341 schools (13.3%) remain used as shelters or otherwise nonoperational – including 331 public, 66 private, and 49 technical institutes – substantially reducing available education infrastructure and safe learning spaces [999][993]. UN agencies have reported widespread physical damage to education infrastructure: UNESCO estimated around 500,000 children forced out of classrooms and more than one million learners affected, with 17 schools destroyed and over 100 damaged, while a UNDP-led geoAI rapid assessment identified 146 destroyed and 264 partially damaged buildings in Beirut and Mount Lebanon [992][1002][993]. Temporary Learning Hubs (TLHs) and examination hubs have been established to preserve continuity of learning – the Ministry of Education (MEHE/CERD) ran exam hubs in four priority locations (Aley, Chouf, Saida, and Beirut) enrolling approximately 600 Grade12 students – and the sector plans to scale up TLHs ahead of the new school year [999][991]. Key barriers to restoring functional learning environments include overcrowded shelters, poor internet connectivity, lack of devices, and limited learning space, which are driving learning loss and increasing protection risks for girls and children with disabilities [993][991][999].

Access to Education and Barriers

Ongoing insecurity, repeated evacuation orders, and mass displacement have severely disrupted access to schooling for children across Lebanon. UNICEF reported that by April the conflict had affected nearly 265,000 students, while UNESCO estimated up to 500,000 children forced out of classrooms [993][992]. As of 8–11 June, Education Sector partners reported reaching 262,384 displaced children through formal modalities – 94,073 inperson and 163,420 online – plus approximately 4,910 children through nonformal activities [991][999]. Key barriers to enrolment and attendance include overcrowded shelters and shared learning spaces, poor internet connectivity, lack of devices, and limited learning space, producing significant learning loss and heightened protection risks particularly for girls and children with disabilities [993][991][999]. Eroding household purchasing power and affordability challenges, together with connectivity and device shortages, are further constraining families' ability to support children's continued attendance and limiting the scale and timeliness of education interventions [991][999].

Education Needs and Current Response

Urgent needs include school rehabilitation, expanded temporary learning spaces, provision of devices and connectivity, targeted psychosocial support, and inclusive accommodations for children with disabilities [992][993]. In response, humanitarian actors have continued delivery of inperson and online formal and nonformal learning, with sector plans to expand TLHs ahead of the new school year to prevent prolonged learning disruption [999][991]. Persistent funding shortfalls and access constraints are limiting the scale, timeliness, and geographic reach of education interventions, leaving underserved areas and displaced cohorts at heightened risk [991][1002].

Health

Availability and Access to Healthcare Services

Health facility capacity has been severely reduced by repeated attacks since 2 March: around 17 hospitals have sustained damage, 42–44 primary health care centres have closed, and at least three hospitals remain fully non-operational, with several unable to resume secondary or maternity services [1035][1041]. Attacks on health care have continued despite ceasefires, causing direct casualties among health workers – including 39 injured at Jabal Amel Hospital on 1 June – and contributing to workforce shortages and reduced service availability [1035][1041]. Over 130 ambulances and emergency vehicles have also been damaged, further hindering timely emergency response and patient transport [1039].

Maternal health access is acutely constrained: UNFPA estimates approximately 16,000 pregnant women among displaced populations (roughly 1,800 births expected per month), while WHO estimated around 13,500 displaced pregnant women in shelters with approximately 1,500 expected deliveries in the coming month, and multiple hospitals remain non-operational or restricted for deliveries [1043][1034][1035].

Health Sector partners have delivered roughly 387,000 consultations for displaced people and provided medications to about 142,600 patients through some 197 PHCs [1048][1034][1041]. WHO and partners have distributed emergency trauma and surgical supplies – including 120 trauma backpacks, 20 trauma/surgery kits, and 15 ICU ventilators – and shipped over 134 m³ (approximately 3.5 tonnes) of medical supplies to frontline facilities [1048][1034][1041]. WHO is also procuring generators and UPS units (due within weeks) and NCD supplies (expected by late June) to support facility resilience [1034][1045].

Critical shortages and stock-outs persist across medicines, EmONC, and trauma items; sexual and reproductive health services remain limited; and specialised care is reduced, prompting urgent calls to replenish supplies and scale up emergency services [1034][1035][1033]. Affordability and transport remain major barriers: although MoPH and partners have expanded subsidised or free PHC services and waived emergency room fees, many patients still incur significant out-of-pocket costs, humanitarian access to hard-to-reach areas is constrained by security concerns and cancelled convoys, and rising fuel and transport costs further limit patients' ability to reach care [1035][1036][1048][1063]. Funding shortfalls are constraining service continuity and scale-up: the Health Sector appealed for an additional USD 61.5 million under the Revised Flash Appeal, with only USD 12.8 million currently available, and overall Flash Appeal coverage remains low, limiting partners' capacity to sustain lifesaving services [1033][1041][1036].

Disease Outbreaks

Communicable disease risks in Lebanon increased in early June 2026 amid massive displacement and strained WASH and health services, with nearly one million people displaced and the majority hosted outside formal collective sites, creating overcrowded conditions that heighten outbreak risk [1058][1053][1038]. Attacks on health care since 2 March have continued to erode system capacity – with WHO recording around 202–203 attacks and multiple PHC and hospital closures alongside hundreds of health-worker casualties – significantly reducing service availability for prevention, surveillance, and treatment [1034][1033][1042].

Shelter- and community-based surveillance has been scaled up but remains inadequate relative to risk. IOM reported strengthening community-based surveillance with 141 reports submitted to the Ministry of Health, while WHO supported activation of shelter-based monitoring and surge staffing for the national hotline to improve early detection [1053][1034][1041]. A specific exposure event of concern involves at least 32 reported rabies exposures in recent weeks, with planned distribution of rabies awareness materials to all shelters to reduce transmission risk [1041].

Response measures in early June included WHO provision of trauma backpacks, emergency surgery kits, and ICU ventilators to priority hospitals, as well as logistics support for generators and NCD supplies, while IOM and partners delivered frontline health services, border health screenings, and referrals in displacement settings [1034][1053]. The Health Sector and WHO have emphasized urgent priorities for expanded shelter-based disease surveillance, risk communication and community engagement, replenishment of essential medicines and trauma/EmONC supplies, and protection of health personnel, while calling for immediate flexible funding under the revised Flash Appeal to prevent further public-health deterioration [1033][1041].

Health System Challenges

Attacks on health care have severely degraded Lebanon's health system: since 2 March, WHO recorded approximately 196–203 attacks, resulting in roughly 131–135 deaths and 379–394 injuries among health workers and patients, and driving widespread facility closures and damage [1035][1042]. Three hospitals remain fully non-operational, 17 hospitals have sustained damage, and over 42–44 primary health-care centres have closed; the attack near Jabal Amel Hospital on 1 June damaged emergency and ICU departments and injured 39 healthcare workers [1035][1059][1042][1043].

Workforce capacity is critically insufficient due to staff displacement, deaths and injuries, psychosocial distress among aid workers, and reliance on volunteers. Facilities are operating at reduced capacity, with increased use of remote and mobile response modalities [1039][1035][1041]. Essential medicines, trauma kits, and EmONC supplies are depleted or in short supply, while fuel and logistics constraints and procurement delays have disrupted supply chains and raised costs [1033][1064][1034].

Humanitarian access constraints – including ongoing insecurity, evacuation orders, route restrictions, UXO contamination, GPS spoofing, and cancelled missions – have delayed or prevented delivery of medical assistance, although 203 humanitarian movements were facilitated through the Humanitarian Notification System between 2 March and 11 June [1048][1042]. Financial shortfalls further threaten service continuity, with persistent medicine shortages and high out-of-pocket costs despite MoPH subsidies, raising acute risks particularly for displaced pregnant women and other vulnerable groups in shelters [1035][1033][1034].

Mental Health and Psychosocial Support (MHPSS)

The National Mental Health Programme (NMHP), co-chaired by WHO and UNICEF, leads a national MHPSS coordination group of 100+ partners. A snapshot covering 2 March–5 June 2026 reported the following service outputs:

- 55,461 people reached with information/awareness sessions or individual consultations including MHPSS
- 28,585 children/adolescents and caregivers reached with awareness sessions
- 58,183 children/adolescents reached with non-specialised/community-based PSS
- 2,938 persons at risk of GBV and survivors accessing GBV response services including PSS

- 11,782 caregivers reached by parenting programmes (7,072 inside shelters; 4,710 outside) [1045]

Clinical and specialist service outputs over the same period included 52 Mobile Crisis Team dispatches (21 hospitalizations), 2,036 specialised consultations with psychiatrists or psychologists, 257 inpatient admissions covered by the Ministry of Public Health across nine hospitals, 43 severe mental health inpatients supported by WHO, 297 centres dispensing psychotropic medications, and 5,111 service users receiving psychotropic drugs [1045]. WHO additionally covered hospitalisation costs for severe mental health conditions for 45 Lebanese nationals, 2 Syrian refugees, and 3 migrant workers since 2 March 2026 [1041].

Hotline and remote support channels were expanded: the 1564 national lifeline recorded 4,832 calls in the MHPSS Coordination Group snapshot to 5 June 2026, and WHO reported 5,481 cumulative calls since onset through 3 June 2026, while WHO also supported surge staffing on the 1787 MoPH hotline [1045][1041]. Capacity-building efforts included training 2,734 frontline actors and 656 participants on standardised procedures for psychotropic medication management, with 63% of PHCCs providing psychological care and 23% providing psychiatric care [1045]. WHO additionally trained 250 health workers across 29 provinces, facilitated 87 peer-support sessions reaching approximately 4,700 health workers, and provided individual psychological support to around 300 health workers while supporting MHPSS integration into primary health care services [1034].

Sub-national MHPSS coordination meetings were held in Bekaa, South, and North governorates, and partners continue multi-sectoral integration of MHPSS into protection, child protection, and GBV responses [1045]. UNHCR and partners provided MHPSS to nearly 8,700 individuals through collective shelters, community development centres, and outreach, while IOM reported reaching 4,301 people with MHPSS since 2 March 2026 and planned community-based MHPSS for 20,000 individuals under its Flash Appeal contributions [1061][1053][1058]. WHO and Health Sector rapid assessments in May–June 2026 highlight increasing mental health and psychosocial needs amid attacks on health care, facility closures, and widened gaps between needs and system capacity, underscoring urgent funding and scale-up requirements for MHPSS across health, protection, and shelter settings [1033][1034].

Health Needs and Current Response

Rapid health assessments conducted in May–June 2026 found a widening gap between rising health needs and declining system capacity, driven by a high burden of non-communicable diseases, increasing mental health and respiratory conditions, constrained sexual and reproductive health services, medicine shortages, workforce shortages, infrastructure damage, and reduced specialised services [1033][1034][1035][1036]. Approximately 134,800–135,000 internally displaced people remain in collective shelters as of 8–11 June 2026, with shelter capacity critically stretched and communicable disease and protection risks elevated [1034][1038].

The most urgent clinical needs are trauma and mass-casualty care and emergency surgery, continuity of NCD treatment, restoration of SRH and EmONC services for pregnant women and newborns, and strengthened communicable-disease surveillance in overcrowded shelters [1034][1033][1043][1036][1048]. UNFPA is prioritising SRH and GBV by deploying mobile medical teams and midwives, supplying Inter-Agency Reproductive Health kits and baby kits, and has appealed for USD 25 million to cover March–August 2026 SRH/GBV needs for targeted populations [1043][1055].

WHO's operational support includes distribution of trauma resources (120 trauma backpacks, 20 trauma/emergency surgery kits, and 15 ICU ventilators), delivery of over 134 m³ and 3.5 tonnes of emergency and trauma medical supplies, funding of hospitalisation costs for non-Lebanese trauma patients, procurement of generators and NCD supplies, and training and psychosocial support for health workers [1034][1041][1048][1068][1045]. Health-sector partners have delivered approximately 383,366 consultations and medications to around 142,597 patients through PHC networks, while IOM reports supporting 18,394 individuals to access essential health services and strengthening community-based disease surveillance with reports submitted to the Ministry of Health [1036][1048][1053].

Critical gaps remain in geographic coverage, stabilisation of supply chains (essential medicines, trauma, and EmONC items), severe workforce shortages and burnout, and high financial barriers to care. The Health Sector urgently requests flexible funding — an additional USD 61.5 million under the revised Flash Appeal, with only USD 12.8 million currently available — while the overall revised Flash Appeal (June–August 2026) requested USD 639.9 million and had received USD 206.2 million (32.2%) by 11 June, constraining the ability to sustain and scale life-saving services [1033][1036][1041][1042]. Coordination mechanisms involve MoPH, MoSA, and multiple UN agencies and humanitarian partners (WHO, UNFPA, IOM, UNHCR), with sector and sub-national coordination and operational hotlines (1787, 1214, 1564) to support referrals and service navigation [1058][1045][1041].

Nutrition

Availability and Access to Nutrition Services

Life-saving nutrition services are being delivered through primary health care, community outreach, mobile teams, nutrition centres, and collective shelters [1077][1076]. Partners are scaling activities to displacement-affected and hard-to-reach areas, but severe access constraints persist in South Lebanon, Nabatieh and other hard-to-reach locations, limiting outreach and referral pathways [1077][1078]. Conflict-related trade and logistics disruptions have reduced availability and raised costs of therapeutic and specialised nutrition commodities, with supply re-routing reported and an OCHA estimate of up to a 40% price increase for the Nutrition Cluster [1080]. Rising food and fuel prices, mass displacement and disrupted markets are worsening food insecurity and reducing dietary diversity, increasing malnutrition risk among market-dependent and displaced households [1078][1082]. Health-facility closures and workforce shortages—particularly in southern areas and Baalbek—are reducing outreach, stabilisation and referral capacity for nutrition services [1076][1089]. Workforce gaps, reduced health-facility capacity, and severe funding shortfalls further constrain the continuity and scale of nutrition services [1076][1089][1077].

Nutrition Needs and Current Response

Humanitarian nutrition partners are delivering emergency rations, screening, IYCF support and targeted referrals, with active coordination with the Food Security Sector to maximise reach, but persistent funding, supply and access constraints remain primary barriers to scaling prevention and treatment services [1077][1080][1076]. Key operational figures and activities include:

- 682,433 emergency nutrition rations and micronutrient supplements distributed to cover at least one month [1077].
- 21,279 children under five and 4,312 pregnant and breastfeeding women screened for wasting, with referrals for treatment where needed [1077].
- The national IYCF hotline received 697 calls between 1 March and 4 June, with 544 callers referred for specialised IYCF support, and 1,107 IYCF Simple Rapid Assessments were conducted to identify feeding challenges [1077].
- Under the MoPH-led Safe Provision of Breastmilk Substitutes (BMS) initiative, approximately 313–335 infants under one received infant formula support, with 3,161 cans of formula distributed [1076][1077].
- Approximately 15,540 caregivers of children under five received counselling and awareness sessions on IYCF, nutrition and early childhood development, and 10 ECD/Mother-Baby Corners were established to support caregiver-child feeding and care practices [1076][1077].

WASH

Availability of Water Sources

Oxfam's Digital Needs Assessment (April–May 2026) found water trucking was the largest water source overall (39.86%), followed by public networks (24.64%) and bottled water (22.46%) of households. Of surveyed respondents, 52.9% reported insufficient WASH quantity, and 35.51% rated WASH quality as "bad/not acceptable," with approximately 35.6% of displaced respondents rating water quality as bad. Returnees were more likely to use water treatment (12%), with boiling and chlorination being common methods among displaced households [1113]. Qualitative findings highlighted municipal service pressure in Bekaa, a disrupted main water source in Saïde after an electrical cable to a borehole was reportedly stolen, and major drinking-water access concerns for households outside shelters in the South and North [1113]. Public water systems have been failing in conflict-affected areas, increasing reliance on private water trucking that is sometimes unaffordable for IDPs [1120]. Seasonal risk is elevated as Lebanon enters the dry season (June–September): 2025 rainfall was more than 50% below average and 2026 forecasts indicate likely ~10% below-average rains, which could compound conflict-related water access pressures [1120].

Access to Sanitation Facilities and Hygiene Practices

Oxfam's Digital Needs Assessment (April–May 2026) found severe sanitation gaps in displacement settings: 56.93% of sampled latrines were reported non-functional, 15.33% had cleanliness issues, safety concerns affected 10.22% and privacy 8.03%, while accessibility concerns were lower at 3.65%. Shared sanitation in collective shelters was frequently described as unclear, poorly maintained, flooded, or lacking supplies and clear gender separation, with Kfar Habou specifically highlighted [1113][1124]. UNFPA's safety audit of 180 shelters found that 28% lacked adequate space and 20% failed to separate female-headed households from unrelated men, with high-risk WASH facilities frequently lacking gender segregation and accessibility, increasing protection and dignity risks for women and girls [1130]. Overall, documented barriers to dignified sanitation and hygiene include widespread latrine non-functionality and poor maintenance, shortages of cleaning and disinfection materials and menstrual hygiene supplies, unclear gender segregation in shared WASH facilities, and overcrowding in shelters [1113][1130][1140]. WHO-supported hygiene activities included 17 hygiene-related health education sessions reaching 267 participants across five shelters [1133].

Waste Management and Vector-Borne Disease Risks

UNOPS documented that the 2024 conflict in Lebanon generated approximately 14.5 million metric tonnes of debris, a volume that can overwhelm local waste systems and complicate safe rubble management and recycling opportunities [1135]. A national digital needs assessment found increased pressure on municipal solid waste systems in areas hosting large numbers of IDPs, notably Al Ain [1113]. Taken together, large volumes of conflict-generated debris, ongoing displacement-driven waste pressures, strained water services, and shelter overcrowding are increasing risks of water- and vector-borne disease transmission [1135][1113]. WHO reported at least 32 recent cases of exposure to rabies and scheduled distribution of rabies awareness materials to all shelters, while initial shelter assessments (14 visits) have been conducted to mitigate health risks in congregate sites [1133]. Humanitarian responses have focused on emergency water supply, hygiene kit distributions, and risk communication – including rabies awareness – alongside site-level WASH interventions to reduce these risks [1135][1113][1144][1133].

Main WASH Needs and Humanitarian Response

As of 11 June 2026, WASH Sector partners reported reaching 1,074,702 people across collective shelters, host communities, and out-of-shelter settings [1140]. Since 2 March, partners have delivered the following at scale:

- 3,765,725 litres of potable water and approximately 110,770 m³ of water to collective shelters through water trucking [1140]
- 851,000 litres of fuel to water establishments, sustaining public water supply for an estimated 841,128 people across 296 supported water stations [1140]
- 114,273 hygiene kits, 68,705 menstrual hygiene kits, and 1,533 cleaning kits distributed to affected populations [1140]

IOM's March–August 2026 Flash Appeal commits to supporting critical water supply, backup generators, sanitation systems, water trucking, bottled water distribution, household water storage solutions, and hygiene-item distributions in collective sites and host communities [1134].

Targeted hygiene promotion has included 17 hygiene education sessions reaching 267 people in shelters [1144][1133]. However, significant constraints to scaling the WASH response remain: the March–May Flash Appeal was only approximately 60% funded, operational and access constraints persist in southern and hard-to-reach areas, damage to water and transport infrastructure has isolated over 150,000 people, and supply-chain and price pressures continue to affect WASH materials [1144][1140][1120][1141]. Field-level reports also cite site-specific failures — such as a stolen electricity cable disrupting a borehole in Saaide — and local solid-waste and water-access problems that sustain household-level deficits despite emergency deliveries [1113]. Coverage gaps remain spatially uneven, and assessments indicate that shelters continue to exhibit gendered protection risks from inadequate, non-segregated, or inaccessible WASH facilities [1113][1130].

Shelter and Non-Food Items (NFI)

Damage to Housing and Shelter Needs

A UNDP-led rapid assessment for Beirut and Mount Lebanon found 146 buildings completely destroyed, 264 partially damaged, 3,168 housing units destroyed and 4,437 units damaged, with an estimated debris volume of about 648,942 m³ in the assessed area [1178]. Nationally, more than 61,000 housing units were reported totally or partially damaged between March and early May 2026, with earlier CNRS/ACAPS data recording over 37,800 housing units affected between 2 March and 7 April, heavily concentrated in Nabatiye, South, Mount Lebanon and Beirut [1172][1173]. Large sudden movements on 7 June reportedly displaced an estimated 4,000 families in Nabatieh, 30,000 families in South governorate and some 50,000 families (~200,000 people) from Beirut's southern suburbs, many of whom sought refuge in open areas or temporary sites [1174][1193][1173]. As of 11 June, 134,800 displaced people (35,700 families) were registered in 642 collective shelters nationwide, with shelters in Beirut, Tyre and Saida at full capacity and North governorate identified as the primary destination for new arrivals [1176]. The majority of displaced people remain outside formal collective sites, with large-scale out-of-shelter hosting in overcrowded apartments and informal arrangements — estimates indicate roughly 85% of women and girls are living outside formal shelters — while others shelter in open spaces or temporary tented sites [1181][1187]. A portion of IDPs live in tents or informal shelters (surveyed IDP share in such shelter types ~7.44%) [1193]. Displaced populations in collective sites include substantial numbers of vulnerable people — approximately 6,800 older persons (55+), 1,200 people with physical disabilities, 661 with mental disabilities, 386 with hearing impairments and 392 with visual impairments — underscoring needs for accessible and inclusive shelter interventions [1193].

Rebuilding faces multiple constraints: widespread housing, land and property (HLP) issues complicate rubble removal and clearance; structural assessments are required before safe recovery; and reconstruction costs, shortages and inflation in construction materials, and limited national fiscal space raise major financing challenges [1205][1172]. Shelter-sector partners are prioritizing repairs and site upgrades — 341 of 420 prioritized sites completed in one update — and scaling site management to support tens of thousands, while appeals identify urgent shelter funding needs (UNHCR/shelter requirements cited ~USD 58.37 million and IOM planning to target some 105,000 individuals across 250 sites with USD 16 million requested) to sustain cash-for-rent, cash-for-shelter, collective-site maintenance, and core relief item (CRI) distributions [1175][1186][1197]. Damage to community infrastructure further limits options for return and safe re-housing: education partners reported 17 schools destroyed and over 100 damaged in initial assessments, with up to 446 schools non-operational by late May, while attacks and damage to health facilities have forced closures and reduced service capacity across the country [1178][1173][1176].

Living Conditions in Camps or Settlements

As of 11 June 2026, 134,800 displaced people (35,700 families) were registered in 642 collective shelters, with shelters in Beirut, Tyre and Saida at full or near capacity while North Governorate is absorbing new arrivals and authorities are activating additional sites and tented accommodation [1176][1175]. A UNFPA safety audit of 180 collective shelters found severe protection and space shortfalls — 28% lack adequate space and 20% fail to separate female-headed households from unrelated men — with widespread overcrowding, absence of locks and partitions, poor lighting, high-risk WASH conditions, and shortages of women- and girl-friendly safe spaces, all of which increase GBV risks [1196]. Women and girls account for nearly 52% of those in shelters, and protection partners report rising psychosocial distress, particularly among children and caregivers, exacerbating protection, privacy and care burdens amid overcrowding and mounting housing insecurity [1192][1177].

Shelter monitoring highlights the concentration of vulnerable groups inside collective shelters, including approximately 6,800 older people, ~1,200 persons with physical disabilities, 661 with mental disabilities, 386 with hearing impairments and 392 with visual impairments, highlighting accessibility and specialized service needs on-site [1193]. WASH partners report reaching over 1.07 million people across collective shelters, host communities and out-of-shelter settings through fuel provision to water establishments and water trucking to shelters, though significant gaps in sustained water and sanitation services persist [1175][1179]. Shelter-sector repairs and site improvements are ongoing: partners prioritized repairs at 420 sites with works completed in 341 (81%), while UNHCR planned repairs and maintenance in 76 collective shelters, completing works in 46 and continuing in 11, to improve privacy, accessibility and safety [1175][1177].

NFI and Shelter Needs and Current Response

Digital needs assessments and community feedback report urgent NFI requirements including mattresses and pillows, kitchen utensils, hygiene and menstrual health (MHM) kits, diapers, washing machines, food-storage items and seasonal clothing to preserve dignity and household functioning [1174]. Inside collective shelters since 2 March, partners report distributions of 111,370 mattresses, 138,569 blankets and 74,350 pillows, with all households in collective shelters having received initial CRIs [1175]. Outside collective shelters, partners have provided targeted in-kind support to vulnerable households, including 27,712 mattresses and 40,229 blankets [1175]. Overall, more than 87,000 displaced people in collective shelters received over 223,700 CRIs, with an additional ~14,000 displaced people in host communities assisted with more than 48,700 CRIs [1177][1183].

Operational modalities are adapting to the scale and diversity of displacement: in-kind CRIs and shelter repairs are prioritized inside collective sites and in hard-to-reach areas, while cash-for-rent and cash-for-shelter assistance is being scaled for households outside shelters, including a one-off shelter cash payment of USD 250 provided to 901 vulnerable families [1175][1197][1179][1183]. Site-management scale-up includes the deployment of mobile SMC teams aiming to support approximately 80,000 people across ~400 sites to strengthen on-site coordination, occupancy tracking and referral mechanisms [1175][1179].

Critical gaps remain across the response. Shelter capacity is stretched, NFI stock and coverage are insufficient for non-Lebanese and other vulnerable groups, and major funding shortfalls persist — the 2026 Flash Appeal reached roughly 60% funding (US\$185.9m of US\$308m), the revised inter-agency appeal requests an additional US\$331.5m, and UNHCR is seeking US\$94.2m to sustain protection, shelter and CRI responses [1187][1186][1183]. Logistical and access constraints driven by insecurity, damaged transport infrastructure (including destroyed bridges isolating populations), and restricted access to some areas are further impeding timely NFI and shelter deliveries, driving the prioritization of in-kind assistance where markets are constrained [1173][1175]. Despite large-scale distribution and repair efforts, gaps remain in coverage, shelter quality, privacy and safety — particularly for non-Lebanese displaced persons facing accommodation shortages and access restrictions — underscoring continued needs for NFIs, shelter maintenance, site management and flexible funding [1173][1196][1175].

Protection

Impact on civilians

The humanitarian impact on civilians has been severe: at least 3,711 people were reported killed and 11,483 injured due to hostilities in Lebanon as of 11 June 2026 [1211]. WHO's surveillance documents widespread, repeated attacks on health care, recording 203 attacks that resulted in 135 deaths and 394 injuries among on-duty health workers, the closure of dozens of primary health centres, and three hospitals remaining fully non-operational while 17 sustained damage, severely restricting access to care [1211][1223][1230]. Health partners and rapid assessments warn of a widening gap between rising health needs — spanning trauma, NCDs, mental health and SRH — and declining system capacity due to damaged facilities, workforce losses, medicine shortages and funding shortfalls, increasing risks to civilian health outcomes and safe deliveries [1223][1236].

Nearly one million people remain internally displaced, with roughly 134,800 people (about 35,700 families) registered in collective shelters as of 8–11 June 2026, placing severe strain on shelter capacity [1250][1211][1222]. Large new displacement movements on 7 June included preliminary estimates of roughly 50,000 families (~200,000 people) displaced from Beirut's southern suburbs and substantial movements in South and Nabatieh governorates (approximately 30,000 families in South and 4,000 families in Nabatieh), overwhelming local reception and prompting temporary evacuations of some collective sites [1222][1232]. Women and girls account for about 52% of people in collective shelters, increasing gender-specific protection needs in congested sites [1232][1211].

Education has been heavily disrupted: attacks and damage to schools affected an estimated 265,000 students by April, and by 28 May around 446 schools were non-operational, removing large parts of education infrastructure from use [1216][1229]. Displaced refugees and migrants face restricted access to shelter and services, growing legal insecurity and loss of documentation, while UNHCR and protection partners reached hundreds of thousands with information, case management and legal assistance and supported birth registration and residency cases in May 2026 [1216][1209]. Economic coping and protection pressures are widespread: Oxfam's digital needs assessment found 32.26% of households reporting debt as a negative coping mechanism (predominantly displaced households), driving eviction risk, landlord disputes and psychosocial distress, and protection partners note constrained GBV and specialised services amid resource shortfalls [1213][1229].

Factors Driving protection risks

According to ACAPS, intensified airstrikes, ground operations and repeated evacuation orders since early March — including the 7 June strike on Dahiyeh that expanded the geographic scope of attacks — have driven civilian casualties and mass displacement, underpinning most protection risks [1212]. Large-scale displacement disproportionately affects women and girls: ACAPS estimates approximately 620,000 displaced women and girls since 2 March, with 85% living outside formal shelters, while the vast majority of all IDPs remain outside formal sites, increasing exposure to protection harms and limiting access to services [1212][1232][1234]. This concentration of risk is compounded for persons with disabilities, older people and those with chronic conditions, raising GBV, psychosocial distress and access barriers [1212][1232][1234].

Severe economic stress is a key driver of protection risks: Oxfam's digital needs assessment found 32.26% of households reporting debt as a coping mechanism, with 59% of key informants linking rent-related debt to eviction risks, fuelling exploitation, child labour and transactional sex [1213][1217]. Humanitarian agencies report major disruptions to health, protection and education services due to damage and attacks on infrastructure including hospitals and schools, constraining survivors' access to medical care, GBV services and legal remedies [1228][1253][1216][1229]. Non-Lebanese populations face restricted access to shelter and services and heightened risks of arrest or detention, while loss of civil documentation and degraded justice and protection systems limit reporting and legal recourse for rights violations [1216][1243][1212]. Overcrowding, shelter shortages and service gaps are driving rises in GBV, trafficking and other exploitation, with UNFPA, IOM and other protection actors warning of sharp increases in these risks unless protection and multisectoral assistance are scaled up immediately [1216][1215][1221]. Humanitarian partners also report UXO and explosive hazard contamination — particularly south of the Litani — impeding safe access, rubble clearance and returns and adding explosive-remnant risks for civilians and responders [1229][1260].

Psychological and mental health impacts

Humanitarian partners report a sharp rise in mental health and psychosocial needs since the escalation began in March 2026, with increasing presentations of anxiety, depression and conflict-related stress across displaced and host communities [1223][1216]. An estimated 770,000 children are experiencing heightened distress, with caregivers reporting extreme fear, nightmares, sleeplessness and feelings of hopelessness, while reported behavioural and coping manifestations include acute traumatic stress reactions among children (including self-harm in case reports), widespread sleep and fear disturbances, and broader negative coping strategies linked to economic strain and displacement [1212][1216][1225][1213].

The National Mental Health Programme (NMHP), co-chaired by WHO and UNICEF and involving 100+ partners, is coordinating a national MHPSS response (snapshot period: 2 March–5 June 2026). Key service delivery figures include: 63% of PHCCs providing psychological care and 23% providing psychiatric care; 2,734 front-line workers trained; 2,036 specialised consultations; 26,777 non-specialised psychosocial contacts; 5,111 people receiving psychotropic medications; 52 mobile crisis team dispatches (21 resulting in hospitalisations); and 4,832 calls to the national lifeline 1564. Digital self-help uptake included 504 new Step-by-Step users and 1,157 Self-Help Plus users [1245]. WHO additionally trained approximately 250 health workers across 29 provinces, conducted 87 peer-support sessions reaching approximately 4,700 health workers, provided individual psychological support to approximately 300 health workers, and supported training of thousands of nurses in emotional crisis management and self-care [1231][1232].

Access to MHPSS is constrained by repeated attacks on health care and disrupted services — including closures of dozens of PHCCs and several hospitals and health-worker casualties since 2 March — while the 2026 Flash Appeal remains substantially underfunded, limiting scale-up of MHPSS services (IOM/MHPSS line: USD 1.8M for 20,000 people) [1211][1223][1230][1250]. Displaced and other vulnerable groups face disproportionate psychosocial impact, with shelter monitoring identifying older people, persons with disabilities and people with mental disabilities among those with elevated barriers to services [1230][1211][1232].

Protection Needs and Current Response

Collective shelters are dangerously congested, lacking basic supplies and privacy, which increases protection risks including gender-based violence. As of 8–11 June 2026, 134,800 people were registered across 632–642 collective sites [1221][1222][1227]. Assessments confirm protection is the largest reported need among displaced populations: Oxfam's April–May 2026 digital needs assessment identified protection as a top concern, with eviction or lack of shelter access affecting 7.26% of households and debt as a widespread negative coping mechanism, underscoring demand for cash, legal aid and targeted community outreach [1213].

Protection actors have reached large numbers: UNHCR and Protection Sector partners reported reaching over 242,500 individuals with targeted protection interventions since the renewed escalation, with activities benefiting 249,505 IDPs and other affected people (62% women and girls) as of 8 June [1209][1222]. UNHCR and partners provided emergency case management to 1,005 individuals (including 179 children), psychosocial support to nearly 8,700 people, and emergency protection cash assistance to 13,550 displaced refugees [1209]. IOM has delivered protection assistance to 7,019 individuals, referred over 3,327 people to multi-sector services, and reached 4,301 with MHPSS since 2 March [1215]. The broader protection response includes legal assistance, case management, cash-for-protection, mobile protection teams, and strengthened site-management and referral pathways [1250][1230][1215].

UNFPA is leading community outreach, GBV awareness and prevention, and dignity-kit distribution (with an estimated gap of 28,000 kits), and is hosting inter-agency PSEA and AAP coordination to strengthen collective accountability to affected people [1221]. Gaps in GBV and specialised protection support are widening amid severe funding shortfalls, with the 2026 Flash Appeal only around 60% funded at the initial stage, leaving major resource gaps for protection services [1229][1220][1216].

Child Protection concerns

Children in Lebanon face rising physical and psychosocial harm from the March escalation: at least 245 children were killed and 957 injured between 2 March and 4 June 2026, and an estimated 770,000 children are experiencing heightened distress from repeated exposure to violence, loss and displacement [1216][1212]. By 7 June an estimated 1.5 million people had been displaced across Lebanon, driving repeated displacement of children and prolonged disruption to safe routines and services [1216]. Schools have been widely affected — nearly 265,000 students were impacted by April and about 446 schools were non-operational by 28 May — heightening protection risks for children, as many school buildings have been repurposed as collective shelters [1216].

Negative coping mechanisms are on the rise among displaced households, with 32.26% of surveyed households reporting debt as a primary negative coping strategy (displaced households accounting for roughly 75% of those cases), alongside increasing reports of child labour and early marriage risks [1213][1209]. Family separation and acute protection cases are also evident: 1,005 individuals at heightened risk received emergency case management, including 179 children requiring follow-up for serious vulnerabilities such as separation, exploitation or violence [1209]. Documentation barriers compound these risks, though UNHCR continued support efforts — assisting 221 refugee children with birth registration and 144 refugees to access legal residency in May 2026 [1209].

Child-focused mental health and psychosocial support has been scaled up: 28,585 children, adolescents and caregivers were reached with child-protection awareness sessions and 58,183 children and adolescents received non-specialised and community-based psychosocial support, while MHPSS integration in the protection sector reached 55,461 persons overall [1245]. Education-sector responses have delivered services, with partners reaching 262,384 displaced children through formal modalities (94,073 in-person; 163,420 online) and 4,910 through non-formal education, yet large numbers remain out of school due to infrastructure constraints and connectivity gaps [1229][1216]. A GBV safety audit of 180 shelters found 28% lack adequate space and 20% do not separate female-headed households from unrelated men, increasing risks of exploitation, abuse and GBV for children and their caregivers [1221].

Violence against women and girls

Humanitarian actors report sharply elevated GBV risks for displaced women and girls amid overcrowded, privacy-poor shelters and disrupted protection services [1221][1216][1229]. ACAPS and related reporting estimate approximately 620,000 women and girls have been displaced since 2 March, with 85% living outside formal shelters – conditions that increase exposure to exploitation and GBV while disrupted protection and justice services limit reporting and access to assistance [1212][1216]. UNFPA estimates 390,000 women of reproductive age among the displaced, including about 16,000 pregnant women and roughly 1,800 expected births per month, and notes that 632 collective shelters are dangerously congested with a shortage of woman- and girl-friendly safe spaces [1221][1227]. A GBV Working Group safety audit of 180 shelters found 28% lack adequate space and 20% fail to separate female-headed households from unrelated men, highlighting shelter design and management risks that raise GBV exposure [1221].

Protection partners, including UNHCR and 78 Protection Sector partners, report reaching more than 242,500 people with targeted protection interventions. UNHCR and partners provided information and GBV risk-mitigation outreach to over 38,000 people between April and May 2026, while GBV response and psychosocial support services reached 2,938 persons at risk or survivors [1209][1245]. Humanitarian plans prioritise survivor-centred GBV services: UNFPA and partners aim to maintain and expand Women and Girls' Safe Spaces, deploy mobile GBV case-management teams, provide cash assistance, legal and psychosocial support, and train front-line workers to strengthen referral pathways [1221][1227]. Gaps remain significant: reports note widening shortages of GBV and specialised support amid funding and capacity shortfalls, and displacement-linked negative coping mechanisms – including early and forced marriage and transactional and survival sex – are reported to be increasing [1229][1216][1250].

Vulnerable Groups / Special Needs Groups

Unaccompanied and Separated Children

As of 8–11 June 2026, at least 245 children were killed and 957 injured between 2 March and 4 June, and an estimated 770,000 children are experiencing heightened distress from repeated exposure to violence, loss and displacement [1267][1270]. Many displaced children are hosted outside formal shelters in overcrowded apartments and informal arrangements, increasing exposure to exploitation, child labour, early marriage, transactional sex and gender-based violence [1267][1287]. Disrupted protection and justice services, limited shelter capacity and widening GBV service gaps are constraining reporting, referrals and longer-term alternative care options for separated or at-risk children [1267][1269][1286]. Protection partners have scaled child protection and case management services but gaps persist; UNHCR and Protection Sector partners report reaching over 242,500 people with targeted protection – including child protection and GBV – and providing emergency case management to 1,005 individuals, including 179 children [1280]. IOM and UN agencies have delivered MHPSS and referrals at scale – UNHCR partners provided MHPSS to nearly 8,700 individuals and IOM reached 4,301 people with MHPSS since 2 March – while specialised GBV and child-protection capacity remains constrained in many areas [1280][1271][1269].

Pregnant and Lactating Women

UNFPA estimates that among displaced people there are 390,000 women of reproductive age, including approximately 16,000 pregnant women, with roughly 1,800 births expected each month [1290]. WHO estimated about 13,500 displaced pregnant women in shelters, with around 1,500 expected deliveries in the coming month as of 11 June 2026 [1274]. Access to prenatal, delivery and postnatal services is severely constrained after sustained attacks on health care: WHO recorded roughly 203 attacks on health care since 2 March as of 11 June, contributing to the closure of about 44 primary health care centres and three hospitals and leaving at least seven hospitals unable to resume maternity services, which restricts safe deliveries and referral pathways [1270][1286][1275]. Health-sector funding and workforce shortfalls further limit service continuity – only 17% of revised Health Sector funding needs for June–August 2026 have been met, exacerbating gaps in maternal and reproductive care [1297]. Nutrition and breastfeeding support has been scaled up: partners report delivery of 682,433 emergency nutrition rations and supplements, screening of 21,279 children under five and 4,312 pregnant and breastfeeding women for wasting, operation of the national IYCF hotline (697 calls, 544 referred), establishment of 10 MotherBaby Corners, and expanded IYCF counselling and infant feeding support [1286]. Overcrowded and protection-compromised shelter conditions heighten GBV and safety risks for pregnant and breastfeeding women: a GBV Working Group safety audit of 180 shelters found 28% lacked adequate space and 20% failed to separate female-headed households from unrelated men, while broader estimates indicate some 620,000 displaced women and girls with 85% living outside formal shelters, increasing exposure to protection risks [1290][1277]. Humanitarian responses prioritise SRH and protection interventions including UNFPA-supported mobile medical teams, midwife deployment to PHCs, baby-kit distributions, integrated GBV case management and psychosocial support, alongside WHO provision of trauma and medical supplies and targeted mental health services to sustain maternal care in displacement settings [1290][1274][1275].

Adolescents and Youth

Education access is severely disrupted: Education Sector partners report reaching 262,384 displaced children through formal modalities (94,073 in-person, 163,420 online) and 4,910 through non-formal activities, while the Ministry and partners are operating temporary learning hubs to sustain continuity of learning [1269][1286]. About 446 schools remain used as shelters or non-operational, putting a large share of education infrastructure out of use and limiting safe learning spaces for adolescents [1286][1267]. Mental health and psychosocial support services have been scaled: protection and MHPSS integration reached 55,461 people with information and awareness, 28,585 children and adolescents and caregivers with awareness sessions, and 58,183 children and adolescents with non-specialised and community-based PSS [1289]. Health-sector and WHO assessments note rising mental-health needs and constrained capacity to meet demand [1271][1280][1274][1278]. Livelihoods and youth-focused programming are included in appeals and response plans: IOM's revised appeal incorporates emergency livelihoods and cash-for-work activities targeting 14,400 individuals, and UNFPA's flash appeal outlines youth engagement priorities – including support to youth-led action, capacity building

for local youth organisations, facilitator toolkits, and youth-designed psychosocial and recreational activities – to support adolescents and young people in shelters and host communities [1295][1290]. Protection monitoring documents rising protection risks for adolescents, including child casualties, psychosocial trauma, and negative coping mechanisms such as increased child labour, early marriage and heightened GBV risks, which exacerbate vulnerabilities among youth in displaced and host settings [1267][1287][1277]. Coordination structures mainstream youth and community engagement into the response – the GBV Working Group is co-chaired by UNFPA, MoSA and UNHCR, and IOM co-leads site management and promotes community engagement – but documents highlight gaps in resources and scale to fully meet adolescents' education, livelihood and psychosocial needs [1290][1295][1269].

People with Disabilities

Shelter monitoring indicates persons with disabilities remain present in collective shelters, including approximately 1,200 people with physical disabilities, 661 with mental disabilities, 386 with hearing impairments, and 392 with visual impairments, underscoring ongoing needs for accessible assistance and services [1275]. Persons with disabilities face additional barriers to accessing healthcare, medication, WASH and shelter facilities and are exposed to heightened protection and exclusion risks amid overcrowded, insecure and resource-constrained settings [1275][1287][1290]. UNHCR and protection partners have delivered targeted support – including emergency protection cash to 1,029 individuals, community outreach to 925 people, and provision of assistive devices and rehabilitation services to 50 people – to address some urgent needs among displaced persons with disabilities [1280]. Recurrent attacks on health care and resulting closures and damage to primary health centres and hospitals have constrained availability of specialised medical care and MHPSS, while WHO and health partners are providing trauma supplies, supporting mental health treatment costs for severe cases, and operating MHPSS hotlines and services to partly fill gaps [1286][1274]. Humanitarian appeals and sector plans emphasise disability-inclusive measures – mobile medical units, accessible shelter upgrades, targeted cash and assistive-device distributions, and inclusive livelihoods interventions – to improve access and protection for persons with disabilities in the ongoing response [1290][1295].

Elderly

Approximately 6,800 older people aged 55 years and above are residing in collective shelters, indicating a significant presence of elderly among displaced populations [1275]. Older people and those with chronic conditions face additional barriers to accessing health care and medications due to medicine shortages, workforce gaps, facility damage, and high out-of-pocket costs documented by WHO and Health Sector assessments in May–June 2026 [1274][1278][1275]. Overcrowded and inadequately equipped shelters – including poor privacy, insufficient gender-segregated WASH, and limited psychosocial support – increase protection and mobility risks for older persons, while geographic isolation and transport barriers outside shelters further constrain access to services and medication [1287][1275]. Non-Lebanese older displaced persons face additional movement restrictions and legal and documentation barriers that limit their access to services, as reported in protection monitoring [1275][1267]. Shelter and protection actors are implementing accessibility adaptations, partitioning, targeted repair and cash assistance, and UN agencies have called for Demographic Vulnerability Assessments to identify and prioritise older persons in referrals and response planning [1284][1286][1290].

Displaced

Since the March escalation, roughly one million people remain internally displaced in Lebanon, with the vast majority hosted outside formal collective sites and relying on informal hosting arrangements [1295][1271]. As of 11 June 2026, about 134,800 people (35,700 families) were registered in 642 collective shelters, with shelters in Beirut, Saida, Tyre and Mount Lebanon at or near full capacity and collective sites reporting occupancy around 96 percent [1270][1271][1286]. Most displaced households reside in rented accommodation or with host families and face rising rents, depleted savings and negative coping – including reduced spending and increased child labour – increasing pressures on food, WASH and other basic services [1280][1287]. Cross-border movements into the Syrian Arab Republic exceeded 418,000 by 8 June 2026, while returns are largely temporary and unsustainable due to ongoing insecurity, infrastructure damage and limited livelihoods [1271][1294]. IOM and partners estimate over 48,000 displaced migrants and third-country nationals who face language barriers, restricted service access, heightened protection vulnerabilities and reluctance to use government-managed collective sites [1295][1271][1267]. Loss of civil documentation and legal insecurity are widespread among displaced refugees and returnees; UNHCR assisted 221 refugee children with birth registration and 144 refugees to access legal residency in May 2026 [1281][1280]. UNHCR and partners have scaled shelter repairs and site management: repairs were planned in 76 collective shelters (46 completed, 11 ongoing) and shelter actors report 341 of 420 prioritised site repairs completed (81%), while site management scale-up aims to support approximately 80,000 people across 400 sites [1280][1286]. WASH partners reached over 1.07 million people with emergency water, sanitation and hygiene services in early–mid June, while health services are severely strained with WHO supporting trauma care hospitalisations and emergency medical supplies [1269][1274][1267]. Cash assistance has been scaled up, with over 137,000 vulnerable refugees reached by Emergency Multi-Purpose Cash Assistance by May and the Ministry of Social Affairs sharing beneficiary lists for approximately 100,000 households under a blanket E-MPCA distribution as targeting commenced in June [1280][1269]. Protection actors report escalating GBV and safety risks in displacement settings – including overcrowding, lack of privacy, inadequate gender-segregated WASH, locks and lighting – and GBV services and specialised support face widening gaps amid stretched resources [1290][1277][1269].

Minorities

Humanitarian reporting in early June 2026 highlights heightened exclusion and protection risks for minority and marginalised groups in Lebanon – including migrants, persons with disabilities, older people, sexual and gender minorities (SOGIESC), and minority-origin Syrians – amid mass displacement and strained services [1283][1271][1275][1267]. Assessments and briefs report rising host-community tensions and perceptions that displaced groups are affiliated with armed actors, fuelling resentment, discrimination, harassment and localised attacks against displaced populations [1267].

Disrupted protection and justice services have reduced victims' ability to report violations, while UNHCR and partners continue to deliver legal residency and civil-documentation assistance and targeted protection interventions [1267][1280][1284]. Humanitarian actors are prioritising inclusion: IOM is co-leading site management and scaling multi-sector assistance for migrants, and UNFPA and the GBV Working Group are expanding SRH and GBV services — including mobile teams and outreach for women with disabilities — alongside calls for demographic vulnerability assessments to map older persons and persons with disabilities, though access, insecurity and funding shortfalls limit coverage [1271][1290][1284].

Operational Environment

Economy

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